Provider Business Optimization

Time Study Analysis

November 10, 2025

FTI CONSULTING™

Time Study Purpose & Objectives

*Datavant's current provider business model relies on approximately **9,000 hourly employees** to **manually retrieve records** across approximately **1,700 sites***

Purpose

- Analyze the **end-to-end medical records retrieval** process from Intake through QC and Delivery
- Generate insights to **inform capacity planning** and **process improvement**

Methodology

- **14 FTI employees** observed about **4,000 requests** both virtually and onsite, typically in **2-3 hour observation sessions**
- Data captured included eIDs, processor names**, phase timing**, nonproductive time, and **key insights/ observations**

INTAKE	FULFILLMENT	QUALITY CONTROL (QC) & DELIVERY
Receive request, verify compliance, confirm/enter information in HS	*Access EMR, locate and compile requested records, download from EMR*	*Upload and review request information, quality control documents, submit request*

QUANT ANALYSIS OBJECTIVES

- Update productivity metrics to reflect real-life scenarios for the capacity model
- Understand differences in processing time for onsite vs remote workers
- Evaluate efficiency of DAR requests
- Understand how differences in request type, intake type, site, EMR used, etc. impact medical request lead time

QUAL ANALYSIS OBJECTIVES

- Understand main pain points of both virtual and onsite processors to recommend process and product improvements
- Evaluate the efficiency of Datavant's current business model to determine areas of improvement for scalability
- Evaluate adoption and accuracy of current digital solutions to determine where lead time can be reduced

Contents

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After observing 190 processors and meeting with 24 stakeholders, we identified >4,000 pain points which will ultimately inform our recommended solutions to sustain, grow, and transform Datavant's business

Conducted Time Study & Stakeholder interviews

- ✓ 142 virtual and 48 onsite processors time studies
- ✓ 24 stakeholder interviews
- ✓ 560 sites
- ✓ 109 systems

Consolidated Pain Points into Themes

- ✓ 2,100+ Time Study and Stakeholder Interview Pain Points
- ✓ 24 Consolidated Pain Points Themes

Quantified Pain Points through Survey and Consolidated Data

- ✓ Defined frequency of pain point to inform prioritization and urgency of solution

NEXT STEPS

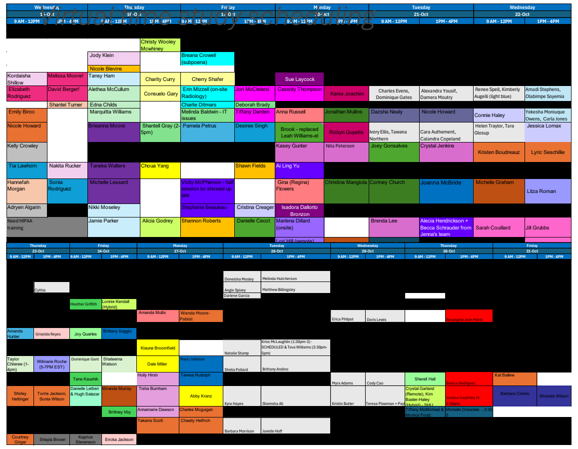
Developed Solutions Aligned to 6 Pillars of Business Model Optimization

- ✓ Unique solutions are being mapped to 6 pillars of business model optimization and categorized into solution areas based on similar elements

Prioritize Solutions and Develop 2026 Plan

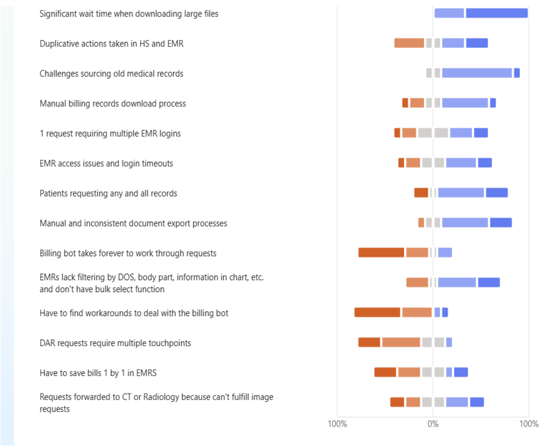
- ✓ Develop solution roadmap and formalize into 2026 plan
- ✓ Identify 5-10 solutions for immediate action

Virtual time study scheduling

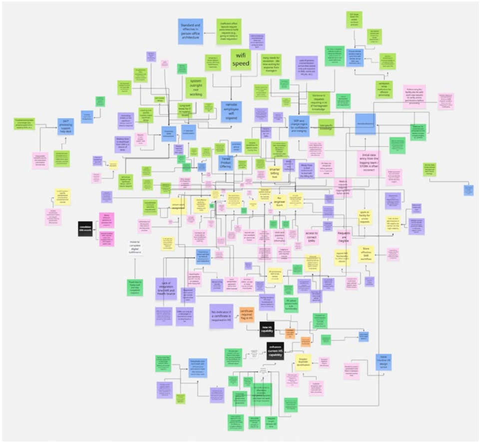


Pain point analysis and mapping

Survey to quantify consolidated pain points



Pain point to solution mapping



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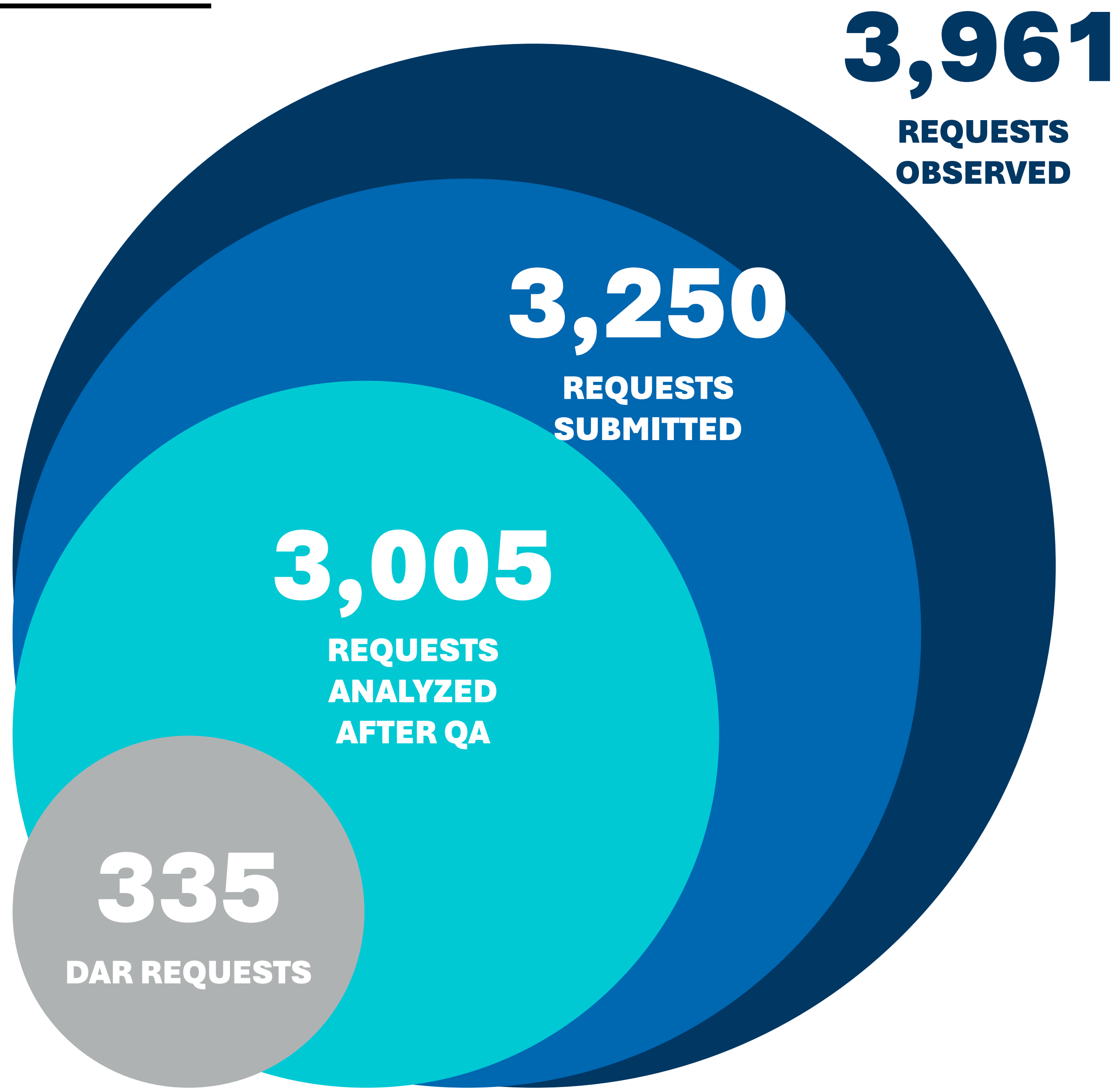
During the two-week time study, we completed 190 observation sessions and observed nearly 4,000 release of information (ROI) requests



18%
NON-SUBMISSION RATE (e.g., pended, escalated)

28%
CL SUBMISSION RATE OF NON-DAR VOLUME

11%
OF TOTAL VOLUME ANALYZED REFLECTS DAR

WHO DID WE OBSERVE?

109 Unique Health Systems

560 Unique Sites

142
REMOTE PROCESSORS

- Access and review medical records
- Extract and verify patient data
- Ensure accuracy and compliance.

48
ONSITE PROCESSORS

- Sort mail and upload site specific requests
- Fulfill requests
- Manage phone and walk-ins

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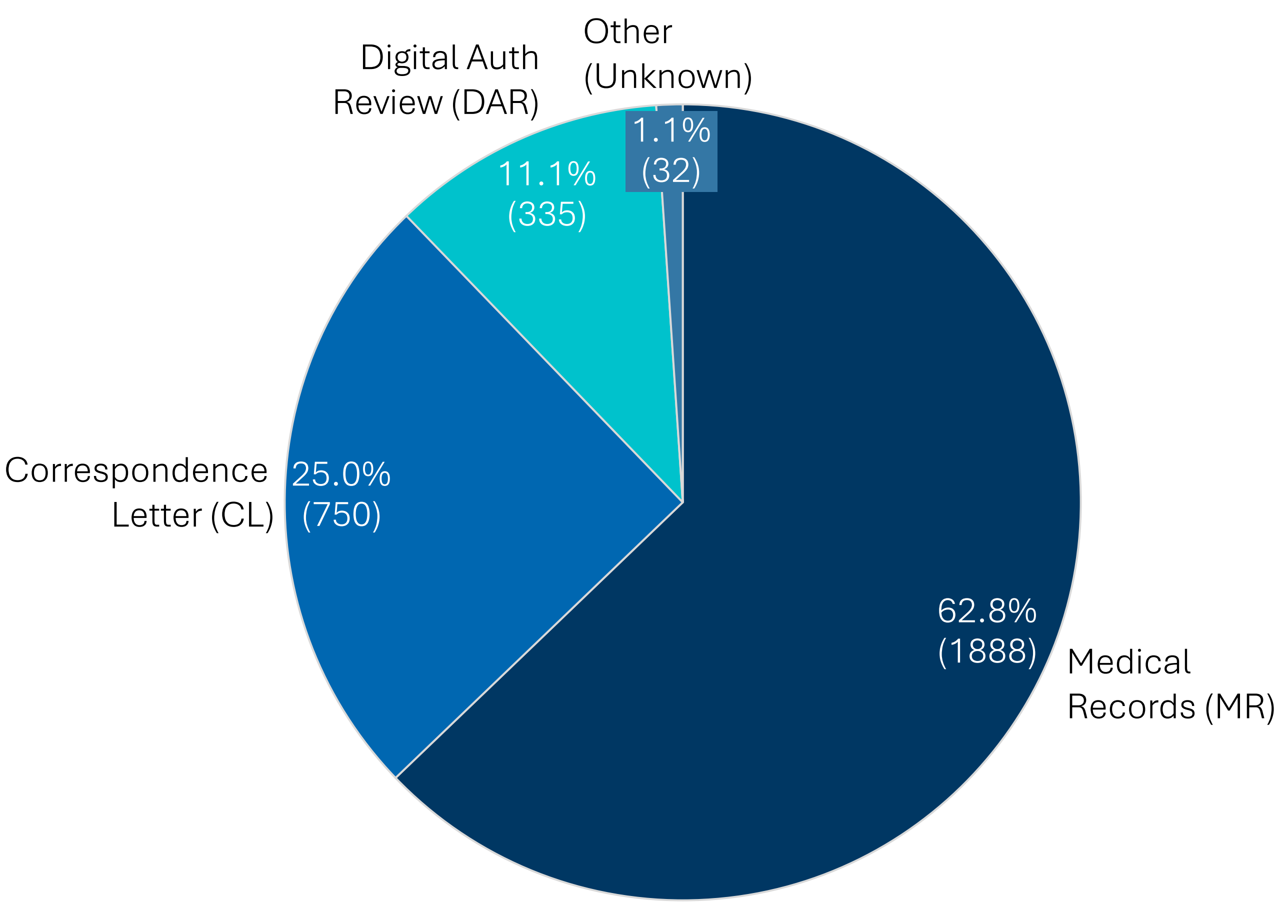
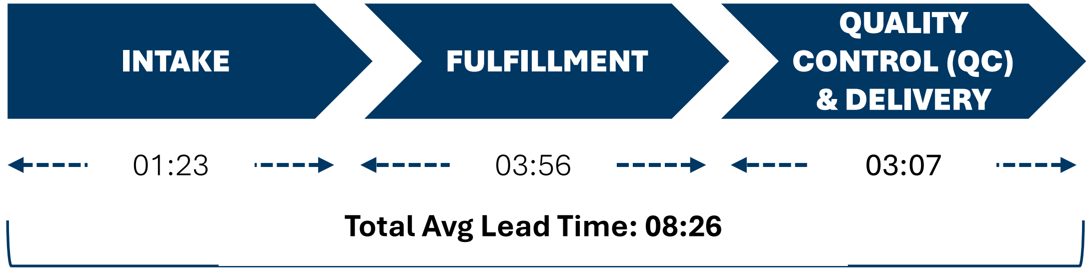
Our analysis was reviewed for quality assurance and focused on three types of requests: medical records, correspondence letters, and DAR

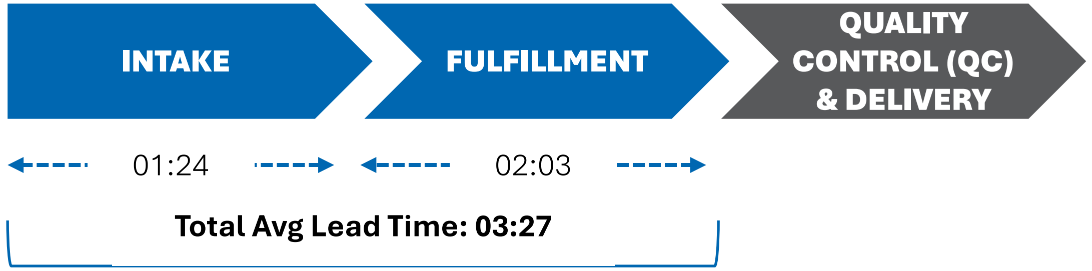
Submitted Sample Size by Record Type



Medical Records (MR) Requests



Correspondence Letter (CL) Requests

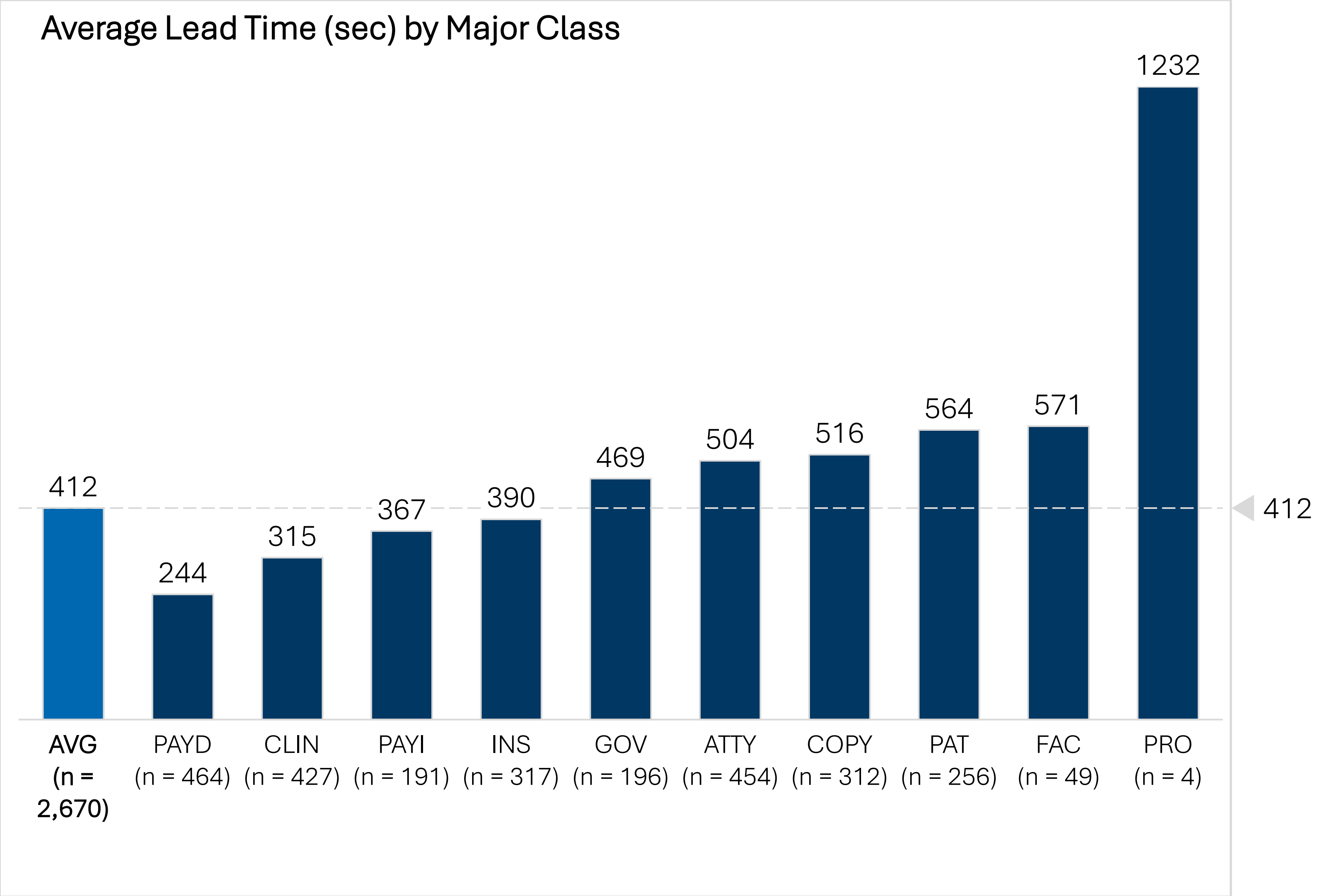


Digital Authorization Review (DAR) Requests*

*Note: Observed DAR requests were focused on the initial approval/denial of the authorization and are not inclusive of any partial or manual fulfillment efforts that may follow.

Lead times are dramatically lower across all major class categories compared to original MAP factors; notably PAT and FAC requests have longer lead times than ATTY and COPY

MAJOR CLASS	ORIGINAL MAP FACTOR STUDY[1] (MM:SS)	Q2 2025 TIME STUDY (MM:SS)	Q4 2025 TIME STUDY[2] (MM:SS)
ATTY	40:00	07:38	08:24
COPY	20:00	07:11	08:36
CLIN*	--	--	05:15
FAC*	15:00	06:13	09:31
GOV	15:00	07:39	07:49
INS	15:00	06:57	06:30
PAT	20:00	09:27	09:24
PAYD	10:00	04:21	04:04
PAYI	10:00	08:48	06:07
PRO	--	--	20:32



**Note: CLIN is a sub-category under FAC. When combined, CLIN + FAC demonstrate average lead time of 05:41 or 341 seconds.*

[1] Original Lead Time Calculations: 60/Requests per Hour. Request per Hour: 6/Original Map Factor
[2] Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

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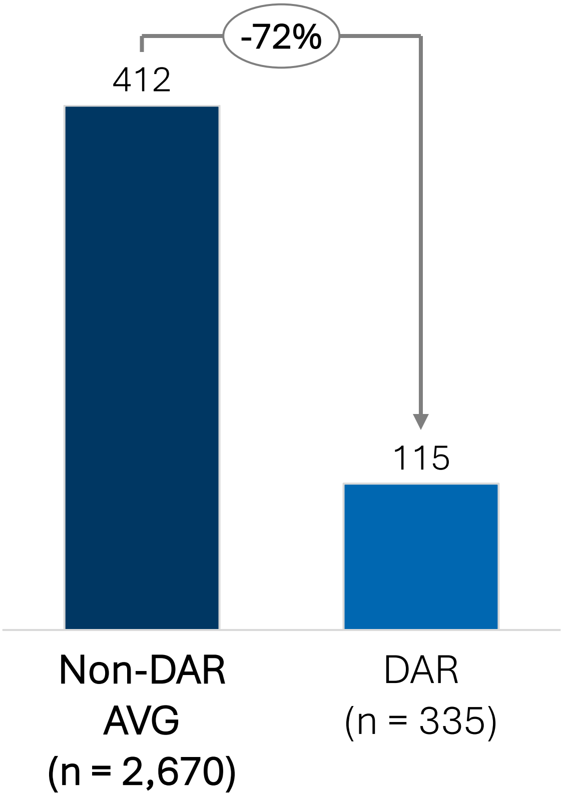
DAR results in faster lead times when digital fulfillment is achieved; however, data suggests that most DAR requests require manual efforts

When the DAR workflow follows the ‘happy path’ for fully digital fulfillment, it results in lead times that are much quicker than the average non-DAR request.

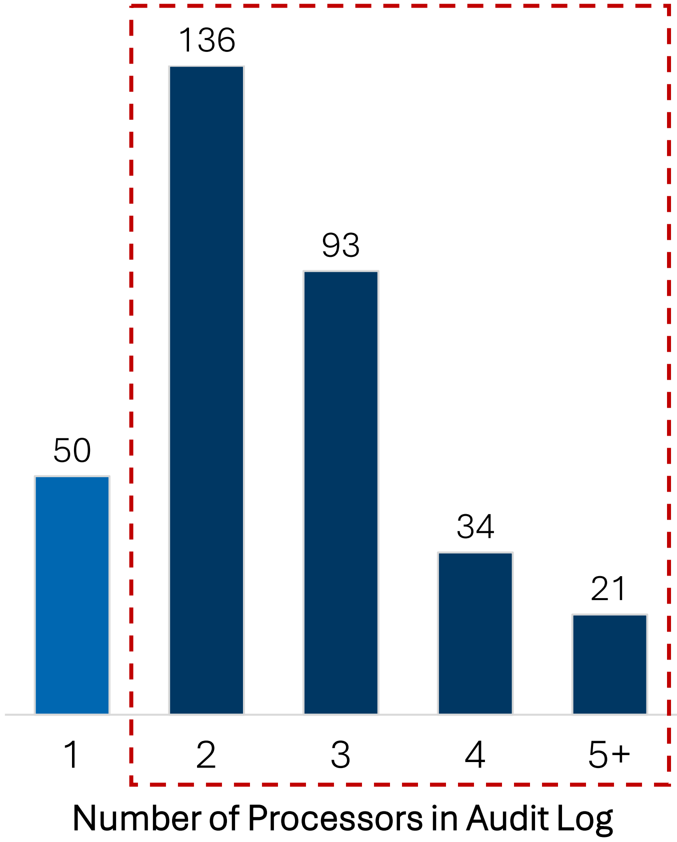
1. Request Intake
2. Logging
3. **Digital Auth Review**
4. Fulfillment
5. Delivery



Automated Step
Manual Step

However, nearly 70% of DAR requests observed may have been sent to partial digital or manual fulfillment, as indicated by 2 or more processors being listed in the request audit log. This extends the overall lead time and implies the limited view during observation was not indicative of total lead time for the request.

1. Request Intake
2. Logging
3. **Digital Auth Review**
4. **Fulfillment**
5. **Delivery**



Automated Step
Manual Step

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Despite attempts to utilize electronic intake types, most requests require significant manual effort during intake and many are unable to be fulfilled

Electronic intake is the most expedient
Major class types PAYD, PAYI, COPY, and PAT handle 40%+ of their intakes processed electronically while other major classes lag in adoption of this method.

37%
OF INTAKES ARE PROCESSED ELECTRONICALLY

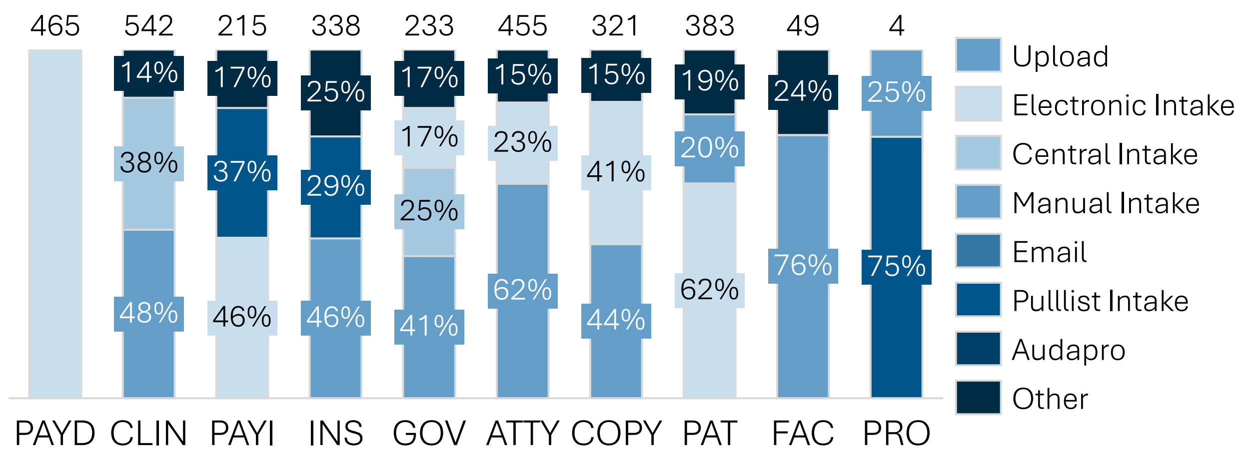
STORK fields require manual revision
During intake, processors often need to update pre-populated fields from STORK to correctly log the request (e.g., dates of service).

27%
OF REQUESTS REQUIRE PROCESSOR UPDATES

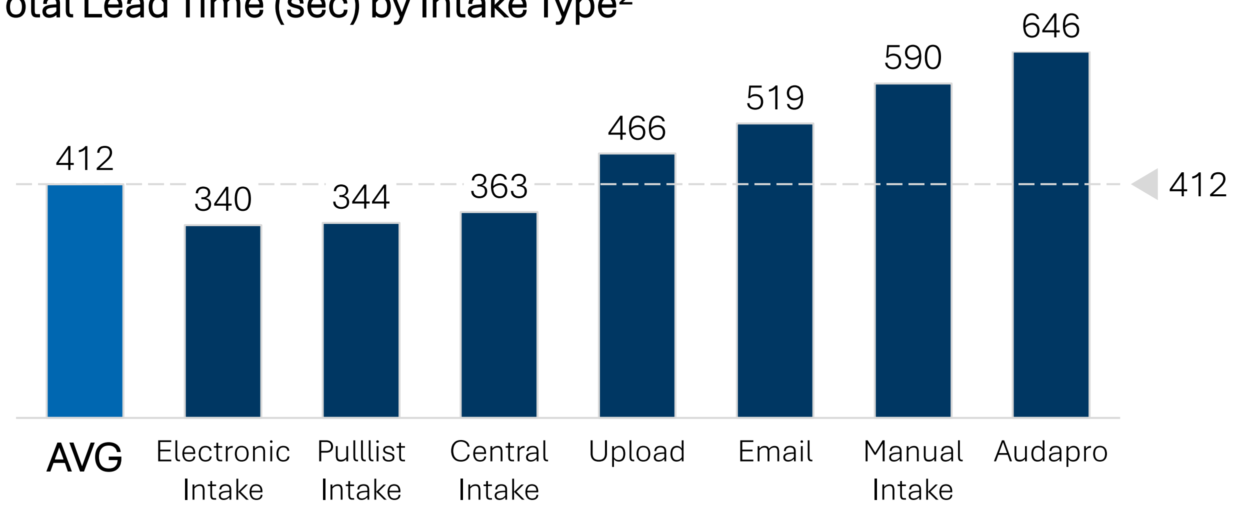
Misrouted requests result in waste
Approximately 18% of requests observed were unable to be submitted due to incorrect site routing or duplicates found in HealthSource (HS).

18%
OF REQUESTS UNABLE TO SUBMIT AFTER INTAKE REVIEW

Intake Type by Major Class[1]



Total Lead Time (sec) by Intake Type[2]



[1] Representative of complete sample size (n =3,005), including DAR
[2] Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Fulfillment is influenced by a myriad of factors including number of EMRs leveraged to access the request, accuracy of initial auth, and request type

Requests from payers and non-clinical third-parties result in more CLs
CLIN, PAT, FAC, and PRO have significantly lower CL rates.

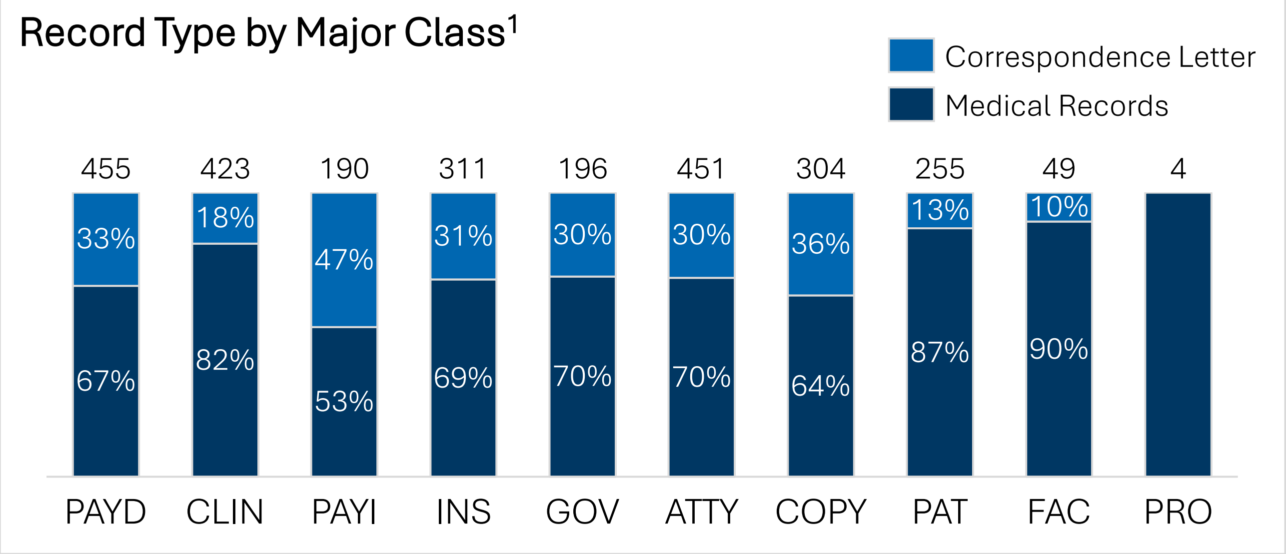
28%
AVERAGE CL SUBMISSION RATE

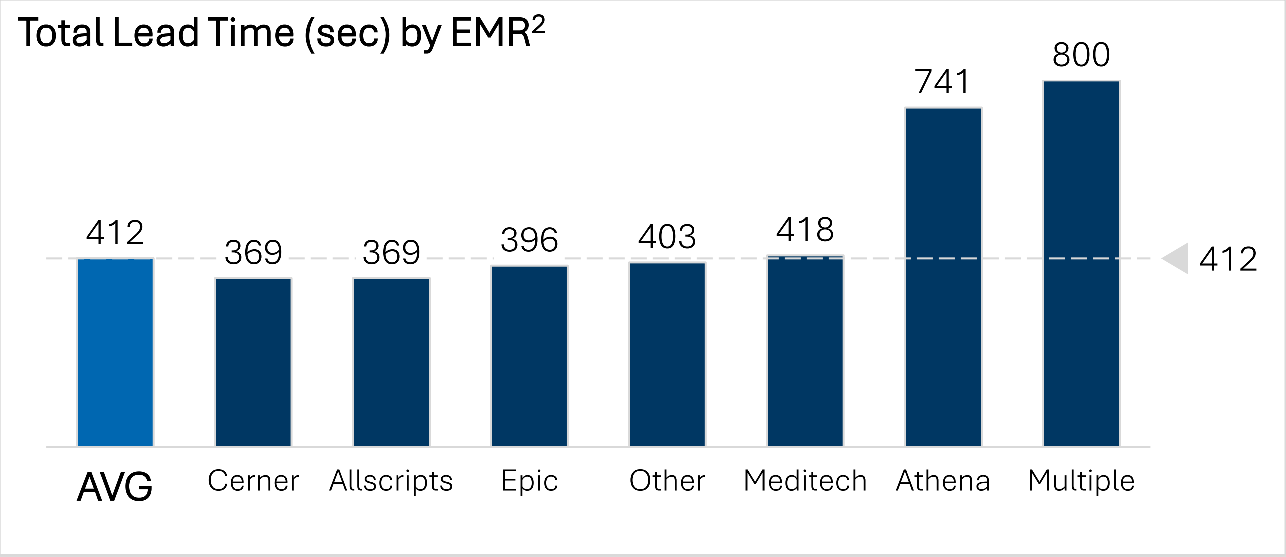
Multiple EMRs slows down processing
The EMR used to fulfill a request is less important than the number of EMRs accessed; multiple EMRs (especially legacy systems) significantly contribute to lead times.

2X
LEAD TIME FOR REQUESTS REQUIRING MULTIPLE EMRs

Audits and certifications are the request types with longest lead times
While the majority of requests are standard (68%), lead times are longer for patient, audit, and certification requests.

12%
OF REQUESTS ARE PATIENT, AUDIT, OR CERT REQUESTS





[1] Representative of non-DAR sample size with blank values excluded (n =2,638)
[2] Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

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A combination of manual QC processes and varying delivery methods influence a significant portion of average total lead times

Page count drives total QC time
QC time represents between 30% and 60% of total average lead time depending on the number of pages in the request.

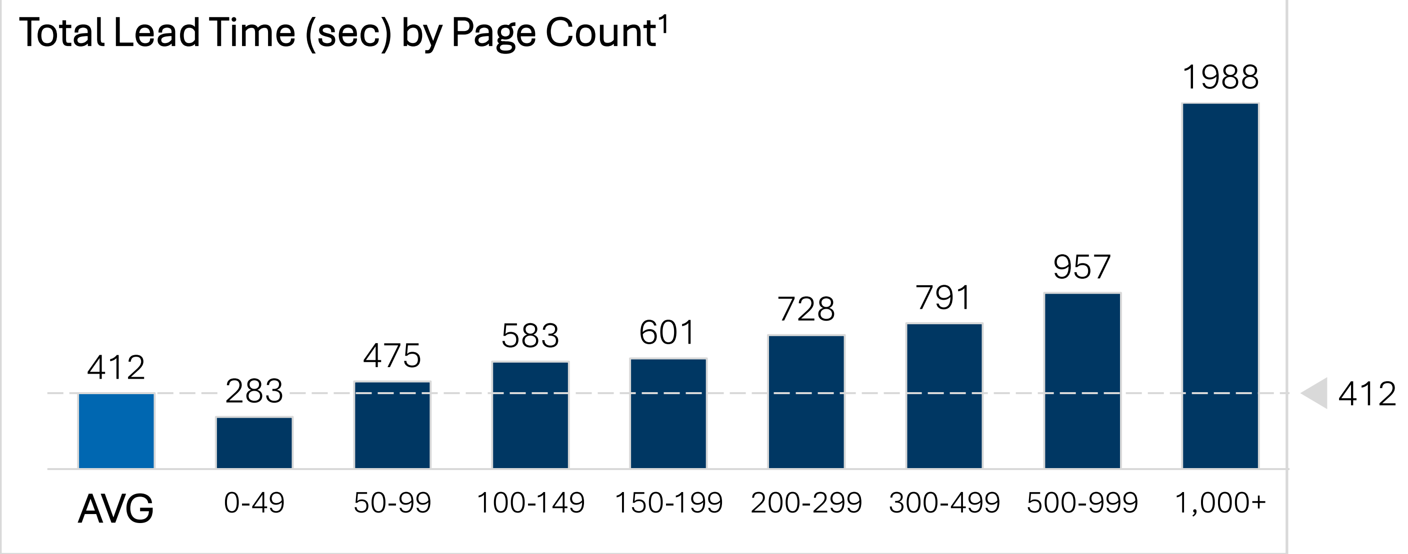
20%
OF OBSERVED MR REQUESTS ARE GREATER THAN 150 PAGES

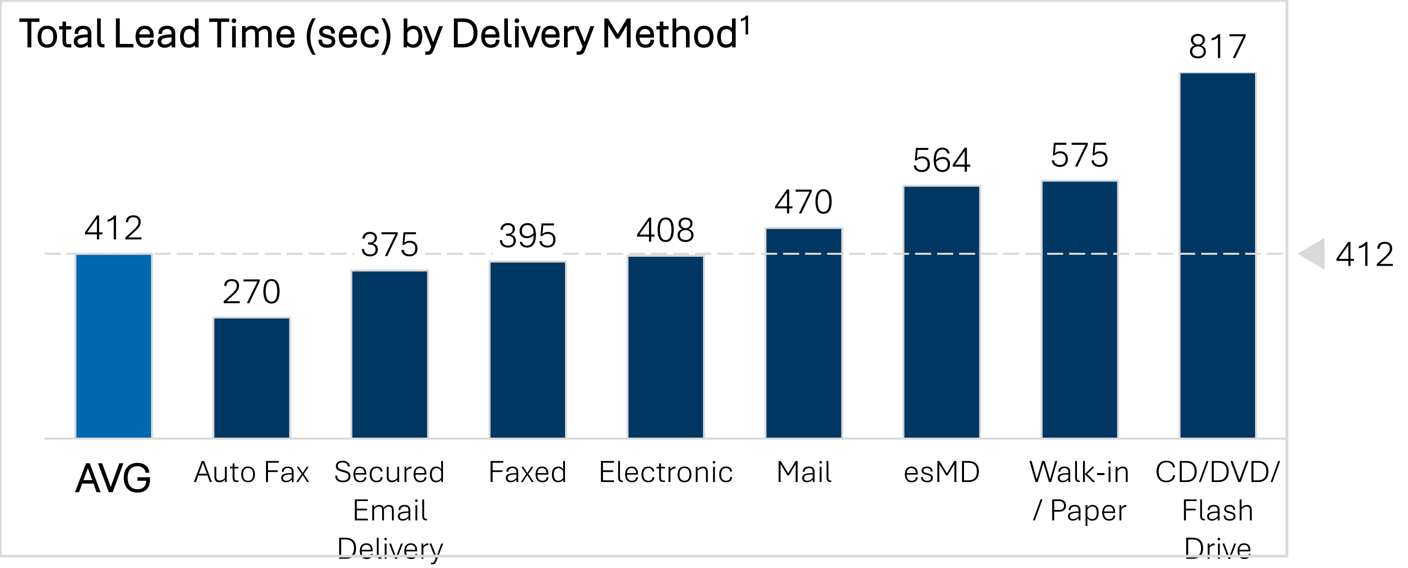
Manual delivery through paper, mail or CD experience the longest lead times
Electronic delivery, except for esMD, have shorter total average lead time.

20%
OF REQUESTS ARE DELIVERED MANUALLY

Manual QC process exposes org to risk
Despite guidance to complete all QC checks during time study for accurate lead times, 11% of requests were still not QC-ed properly according to SOPs.

11%
OF MEDICAL RECORD REQUESTS ARE NOT PROPERLY QC-ED



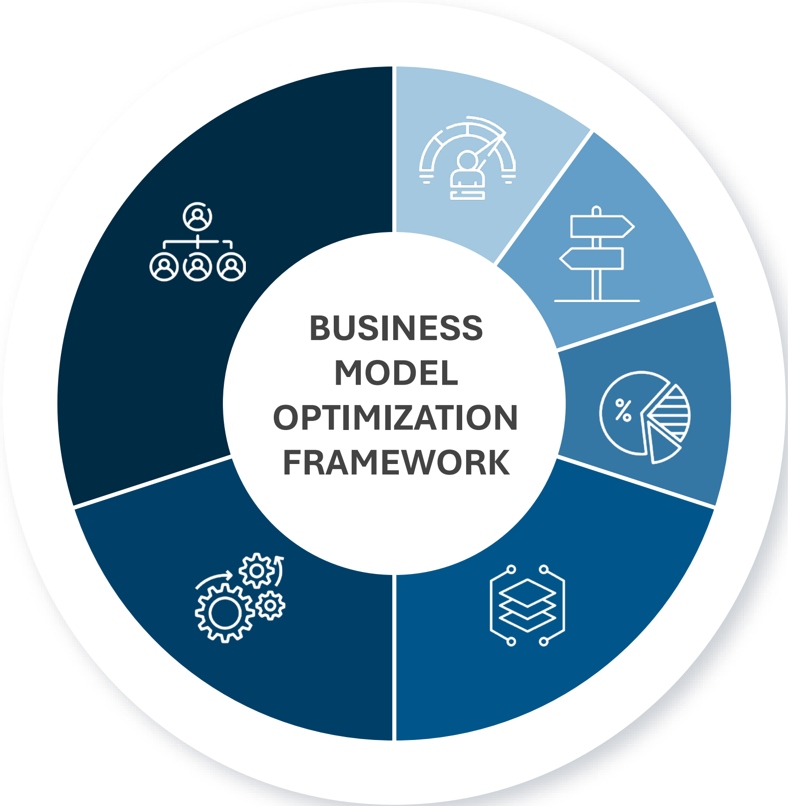


[1] Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Contents

Our qualitative time study analysis focused on identifying key pain point themes aligned with the six pillars of business model optimization

As the provider business shifts from manual, regional operations with high resource intensity to a more digitized and automated model, Datavant's provider business model will need to evolve. There are a ***range of critical pain points*** that need to be addressed to optimize the business model.



1: ORGANIZATION MODEL & RESOURCE PLANNING

- Role scope extends well beyond processing tasks diluting focus and reducing efficiency
- Workforce and volume forecasting processes are manual, do not factor in other productive time and lack integration*
- Excessive request escalations indicate process inefficiencies and gaps in frontline issue resolution
- Decentralized operations have produced fragmented, site-driven processes that limit operational efficiency*

2: OPERATIONAL PROCESSES

- Human-driven processing is time-consuming and requires significant manual effort, training and resources
- Requests often lack information required to fulfill due to lack of education
- Patients or Patient DOS not found in the EMR
- Sub-optimal work setups for remote and onsite impact performance

3: TECHNOLOGY ENABLEMENT

- Outdated or inadequate access to technology needed to get job done
- Tech outright not working and delaying duties by hours or days with support often taking many hours or days to fix
- Ecosystem partners lack integrated features and functionality critical to efficient request fulfillment
- Intake process requires manual updates and STORK is often incorrect
- Prepopulated correspondence letters require manual support
- Duplicate detection logic inaccurately flags distinct requests and true duplicates
- Work assignment processes are ineffective, as the 'Work Next' feature is not consistently used, and each site applies differing method
- The HS system lacks intuitiveness and requires excessive clicks to complete simple tasks
- Requests routed to wrong facilities or processors
- HS's system limitations make it difficult for employees to complete tasks efficiently without relying on manual workarounds

4: GOVERNANCE & COMPLIANCE

- Standardization efforts ineffective due to lack of investment in change management*
- Duplication of efforts, blurred accountability and fragmentation across organization*
- QC process extremely manual, and Verify Assist is often inaccurate

5: PERFORMANCE MANAGEMENT

- Productivity metrics do not reflect actual productivity requiring additional documentation and data analysis to assess and track processor performance*

6: GO-TO-MARKET

- Customer incentives could be better aligned to incentivize shifts to digital and global*
- Fragmented contracting and service models create inconsistent delivery and reactive renewals*

*Indicates theme observed in both exec stakeholder interviews and time study

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1. Org Model & Resource Planning

Disjointed and inconsistent structure prevent optimized resource utilization and efficiency

Role scope extends well beyond processing tasks diluting focus and reducing efficiency

- Many different intake channels (subpoenas, walk ins, emails, fax, mail) creates manual work to intake and log requests that could be diverted to digital channels
- Lack of request-based role specialization creates more time learning how to process each type of request
- Significant time is spent managing and reviewing emails relating to intake and patient / provider comms
- Sorting mail, unstapling, scanning documents is highly manual
- Phone calls from the call center can hinder workflow productivity

Manual mail processing **added ~1 hour to onsite staff workload** inclusive of sorting, reviewing, scanning new mail and shredding old mail

At one site a patient walk-in required **printing a 900-page record that took over 40 minutes.**

Workforce and volume forecasting processes are manual, do not factor in other productive time and lack integration*

- Site realities (walk-ins, phones, mail, on-site interruptions) are not reflected in staffing models
- Workforce and volume forecasting does not accurately capture other productive time outside fulfilling requests (e.g., mail sorting etc.)
- Planning is maintained in local Excel trackers by each region, with no centralized data aggregation or integration of various forecasts to model demand and dynamically align labor to volume trends
- Some sites fulfill records outside of HS (e.g., CoC/stat requests in EMR) and these volumes are only reflected in a single line-item bulk entry rather than the rich detail needed to understand demand

*"The Scheduling Tool might say we need 10 people, but it doesn't fully reflect that three are remote (pure processors) while **seven are on site juggling walk-ins and phones, so their throughput differs**."*
– HIOM –

Excessive request escalations indicate process inefficiencies and gaps in frontline issue resolution

- For remote teams, there is a lack of knowledge about the site requiring frequent escalation / manager involvement
- Processors often escalate even minor questions or doubts and tend to wait for managerial responses instead of proceeding independently
- Limited immediate access to supervisors necessitating escalations
- Unclear guidelines for special cases

Of the nearly 4,000 requests observed in the study, **~18% were not submitted**. A primary driver of non-submission was unanswered open questions resulting in pend status and multiple touches to fulfill.

Decentralized operations have produced fragmented, site-driven processes that limit operational efficiency*

- Onsite roles vary from site to site given SOPs are highly customized to customer / site
- Service creep can tend to arise as Datavant employees become embedded in customer teams and their roles extend beyond core ROI fulfillment
- In the absence of standard SOPs, sites often take on additional tasks as a protective measure (e.g., additional QC checklists and templates)

*"I have a facility where they changed the phone tree. We used to be #4, but now we're #1, so people call in and the call comes to us even when it's not for us. Then **we have to figure out where to transfer them**."*
– HIOM –

Indicates theme observed in both exec stakeholder interviews and time study

2. Operational Processes

Manual processing matched with system limitations slow operations and reduce productivity

Human-driven processing is time-consuming and requires significant manual effort, training and resources

- System fails to identify DAR requests requiring manual fulfillment
- DAR often required multiple touchpoints suggesting digitally eligible requests are frequently required to be either partially or fully fulfilled manually
- In some cases, processors are required to travel to FedEx to mail and ship completed requests
- Billing process is incredibly manual and tedious with little integration preventing effective workflow

Almost **70% of submitted requests are handled by 2 or more processors,** demonstrating the manual effort required to log and fulfill requests.

Requests often lack information required to fulfill due to lack of education

- Requests often lack necessary authorization
- Some requests are handwritten and illegible
- Requests often lack required information to fulfill as evidenced by the 28% of observed requests being correspondence letters
- Outdated letterhead leads to misrouted requests (i.e., sent to the wrong address)
- Requestors frequently request the entire record when only a few service dates are sufficient

On-site observations noted almost **1 in 5 walk-in requests resulted in the processor requesting the patient to complete a new authorization form** when the items they checked off did not align with the verbal request they made.

Patient or date of service cannot be located in the EMR preventing processors from fulfilling requests

- Fulfillment delays occur when patient or encounter information cannot be located in the EMR due to a myriad of reasons
- Patient data found in a Legacy source which is manual / tedious to identify (sometimes taking days to locate records)

About **28% of completed requests observed resulted in a correspondence letter**, indicating gaps in the initial request and authorization

Sub-optimal work setups for remote and onsite impact performance

- Remote: HS/EMR processing and download times are prolonged when using personal Wi-Fi
- Remote: Workplace setup ineffective for efficient processing
- Remote: One laptop screen to manage 3+ programs
- Onsite: office architecture makes it timely to execute tasks (e.g., mailroom on different floor etc.)
- Onsite: hospital affiliated managers determine location of DV employees, sometimes more susceptible to out of scope distractions (e.g., walk-in requests not for records etc.)

5 remote processors observed were working from a single screen, slowing down their processing time exponentially.

*"I am stuck sitting by the walk-in window, and it **causes a ton of distractions as not every visitor is there for me**, but management made me do it."*

– Onsite Processor –

*Indicates theme observed in both exec stakeholder interviews and time study

3. Technology Enablement

Outdated systems and insufficient tools force manual workarounds, reducing productivity and slowing down processing

Outdated or inadequate access to technology needed to get job done

- Lack of access to Adobe causes export compilation difficulties for bulk request upload
- Can't directly print from EMR into HS so have to download to PDF and then upload and most processors don't have Adobe license
- Lack of access to EMR needed to fulfill a request
- Lack of access to paper charts needed for fulfillment

70% of observers noted challenges sourcing old medical records, primarily due to limited access rights for legacy EMRs.

Tech outright not working and delaying duties by hours or days with support often taking many hours or days to fix

- Serious tech issues causing some onsite processors to only get through 2-3 requests / day some days
- Frequently HS is outright not working for hours
- HS is slow, inconsistent, and often fails midway through uploads requiring frequent manual support
- Systems take longer in the VDI
- Significant wait time when downloading large files from EMR
- There are long wait times for IT support — often many hours or even days — and they frequently fail to resolve issues, leaving processors to find their own solutions

During one on-site visit, **the office sat stagnant for 3 hours due to an internet outage and long response time from IT**

7 FTI Time Studies (4%) could not be conducted due to **processor tech issues**

Ecosystem partners lack integrated features and functionality critical to efficient request fulfillment

- EMRs lack filtering by DOS, body part, information in chart, etc. and do not have a bulk select function so processors have to look through the entire patient chart
- No direct HS/EMR integration for documents (users must manually download / upload records)
- No direct HS/EMR integration for data fields (requiring a lot of copying and pasting across systems)
- Processors have to log into multiple EMRs for one request to find data

Many processors who work primarily in Epic commented on **recent changes that were made to the search feature within the EMR that limit their ability to pull records as effectively as before**.

 **Indicates theme observed in both exec stakeholder interviews and time study*

3. Technology Enablement

Current HS features slow operations and force workarounds, highlighting the need for more robust engineering and user experience design

Intake process requires manual updates and STORK is often incorrect

- Stork does not populate information accurately, DOS requested incorrect in nearly every session observed

90% of observed processors needed to update prepopulated STORK information always or often.

About **27% of requests required manual updates to pre-populated logging fields from Stork**. The most commonly updated field was date(s) of service.

Prepopulated correspondence letters require manual support

- CL template has insufficient detail so must be manually revised
- Processors had CL letter templates on digital notes, email drafts, and post-it notes to reference when CL was needed
- CL process varied site to site
- Correspondence workflow is very manual

90% of observers noted additional manual effort for CL's through post-it notes, email drafts and digital notes to modify pre-populated CL templates in most instances of CL delivery observed.

Duplicate detection logic inaccurately flags distinct requests and true duplicates

- Duplicate detection logic inaccurately flags distinct requests, creating unnecessary manual support and confusion
- Duplicate detection logic fails to flag true duplicates, allowing duplicate requests to be processed and causing additional cleanup effort

Out of 186 observed requests that noted the potential duplicate workflow, **62% were false positive requests that were unique and could be fulfilled.**

Work assignment processes are ineffective, as the 'Work Next' feature is not consistently used, and each site applies differing method

- Some processors selectively choose which requests to fulfill to hit metrics
- Lack of consistency across teams and HIOMs for assigning work to team members for the day – email, excel based forms etc.
- Processors often open request to see other processors in the same request
- Work assignment can be ineffective onsite as it does not always accurately consider unpredictable in-person tasks and distractions

During one session, **a processor was assigned 15 requests to work on but after opening HS, half of their requests had already been completed.** Non-productive time followed while the processor asked their supervisor what to work on next.

*Indicates theme observed in both exec stakeholder interviews and time study

3. Technology Enablement

Critical features are missing from HS, creating gaps that slow operations and increase manual effort

The HS system lacks intuitiveness and requires excessive clicks to complete simple tasks

- HS is designed as an inventory system, not a workflow tool, limiting its effectiveness for request fulfillment workflow
- HS user interface and user experience (UI/UX) is inefficient, forcing users to navigate through many clicks to complete common tasks
- Resetting system parameters and filters in between requests wastes time
- Ineffective HS comments / audit log creating excessive scrolling to identify pertinent information

About **40% of observers noted processors wasted time resetting search parameters in between requests.**

Requests routed to wrong facilities or processors

- Re-routing requests to correct departments occurred frequently and required site specific knowledge to determine correct location
- Re-routing requests consumes extra time, requiring phone calls, emails, or faxes to direct them to the correct destination
- Many requests needed to be forwarded to CT or Radiology because processors cannot fulfill image requests
- Realignment of requests often required prior to executing on work queue to ensure all requests are accurately tagged to the processor
- Billing requests often misrouted and need to be rerouted to billing specific processors

68% of FTI observers noted they saw rerouting during their sessions.

HS's system limitations make it difficult for employees to complete tasks efficiently without relying on manual workarounds

- HS provides no indicator when a certificate is required, increasing risk of incomplete processing
- HS requires bulk selection for multiple date ranges (partially or fully fulfilled) with no option for customizing different dates, creating inefficiencies
- Pulling records in 250-page increments for secure capture is extremely time-consuming
- Processors cannot easily identify secure information in HS, forcing manual searches (Ctrl+F) and slowing workflow

About **12% of submitted requests were greater than 200 pages**; this document length **typically resulted in failed uploads and/or needing to break the request into multiple files** to upload to HS.

**Indicates theme observed in both exec stakeholder interviews and time study*

4. Governance and Compliance

Lack of inconsistent compliance increase risk and operational vulnerability

Standardization efforts are limited by lack of investment in change management*

- Leaders repeatedly described "change rolling downhill" without consistent enablement
- Frontline teams are not always aware of or equipped to carry out new workflows and/or benefit from technology rollouts due to a lack of dedicated and coordinated change management efforts.
- There is no single owner for change management creating an accountability gap
- Product, Business Ops, and Operations each take partial responsibility in change management creating duplication and inconsistent adoption

*"We've expanded product and engineering rapidly, which means more change, and we need a scalable operational process for enablement. **Without centralized governance, we don't 'force' adoption; we influence**. **Training variance is a challenge**."*

\- Exec Stakeholder -

Duplication of efforts, blurred accountability and fragmentation across organization*

- The Provider organization operates as a collection of semi-autonomous verticals
- Duplication of effort is common (i.e., Product and Business Ops both drive digitization)
- Accountability lines are blurred, especially across product, operations and business operations

*"We have a RACI within my team, but **fewer cross-team RACIs from the top down.** With thousands of sites, there's **significant variability in how work gets done…it complicates standardization**."*

\- Exec Stakeholder -

QC process is extremely manual, and Verify Assist is often inaccurate

- HS Verify Assist is oftentimes incorrect, completely ignored, and does not work on large requests
- Some processors exclusively rely on Verify Assist for QC, creating compliance risk as Verify Assist is often inaccurate
- Many sites require additional QC forms that drive inconsistent practices and varying productivity levels
- Quality audits are not routinely carried out or defined as a required role

*"Quality measurement needs a more proactive structure. We track UADs and non-UADs, but a single UAD can disproportionately impact a strong associate. Leaders should **conduct standardized monthly audits** per associate and track trends over time to coach proactively, not only react after incidents."*

\- VPO -

**Indicates theme observed in both exec stakeholder interviews and time study*

5. Performance Management & 6. Go-to-market

Across the workforce and customers, incentives need to be aligned to strategic goals

Productivity metrics do not reflect actual productivity requiring additional documentation and data analysis to assess and track processor performance*

- Productivity targets incentivize processors to prioritize high-credit requests, leaving complex or lower-credit requests unaddressed and fueling backlogs
- Metrics do not account for time spent on manual tasks and onsite dynamics such as phones, scanning, or reconciliation, distorting performance and workforce planning
- Productivity tracking varies widely across sites, with some leaders using UPH, others OMAP, and others requests per hour and there is no consistent method to measure time-on-task
- Work done outside HealthSource is manually reconciled into HealthSource creating an "honor system" and potential over- or under volume reporting, obscuring true throughput and distorting productivity trends
- Field and zone leaders' KPIs are operational, not transformational. Most leaders (VPOs, GMs) are measured on OMAP productivity, turnaround time, and margin, not digital or global penetration

*"By looking at both request per hour and OMAP, I was able to identify that **my processors were gaming the system**, so we had to have tough conversation with them and create our productivity goals."*
- HIOM -

Customer incentives could be better aligned to incentivize shifts to digital and global*

- Customers see digitization as Datavant's cost play, the revenue-share model makes it hard for customers to see direct financial benefit from digital fulfillment versus manual
- Outdated contracts and SOPs lock in manual, onsite work. Evergreen terms and pre-digital clauses slow scaling of digital and global models
- Inconsistent contracting and governance create friction. Onsite and remote terms vary, requiring ad-hoc renegotiations and reliance on local relationships
- Client IT resourcing is a major blocker. Security reviews, credentialing delays, and competing priorities often stall digital integration

*"Providers don't necessarily see faster digital retrieval as their problem to solve. They think it's **Datavant's efficiency play, not a value add for them**."*
- Exec Stakeholder -

Fragmented contracting and service models create inconsistent delivery and reactive renewals*

- Customers receive varying levels of support (basic fulfillment vs. full service) without clear contractual differentiation, leading to scope creep and inconsistent services
- Contract data is dispersed across "Member Admin" spreadsheet, Salesforce, LinkSquares, and BI tools, with no unified view of renewals, service commitments, or term expirations
- There's no automated renewal or alert system; renewals rely on individual tracking and relationships, often leaving accounts to lapse or renew reactively

*"**Onsite requirements aren't always contractual**, they're often managed via site SOPs, which are inconsistently maintained. The majority are out of date."*
- Exec Stakeholder -

*Indicates theme observed in both exec stakeholder interviews and time study

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Contents

Emerging solution areas across the business model elements

BUSINESS MODEL ELEMENT	#	SOLUTION AREA
ORGANIZATION MODEL & RESOURCE PLANNING	1	**Executive Role Clarity:** Realign functional responsibilities across business functions (e.g., product, business operations, operations, SAM, etc.) to clarify accountabilities, avoid duplicative functions and create a "connective tissue" to drive cohesion
	2	**Operations Structure & Role Standardization:** Separate account management from operational delivery, clearly define processor roles to distinguish core fulfillment from ancillary tasks, and introduce role specialization (e.g., DAR or contact center) to improve focus and efficiency.
	3	**Workforce Management:** Implement workforce management solution and processes to enable consistent, data-driven forecasting, planning, and optimization of capacity and performance.
	4	**Dedicated Change Management:** Establish dedicated resources to drive and operationalize change across functions, connecting Product, Operations, and Tech, and embedding consistent training, communication, and adoption planning to ensure new processes and tools are implemented effectively.
OPERATIONAL PROCESSES	5	**Service Standardization:** Develop and enforce consistent Standard Operating Procedures tied to tiers of service offerings to reduce variation and improve quality.
	6	**Improve Intake:** Streamline and increase digitized request intake to minimize errors, reduce manual steps, and speed up processing.
	7	**Improve Digital Fulfillment:** Expand and enhance digital fulfillment to enable faster, end-to-end digital record delivery.
	8	**Finance / Revenue Ops Transformation:** Improve financial forecasting, reporting and revenue recognition
TECHNOLOGY ENABLEMENT	9	**Stabilize & Optimize ROI Technology:** Strengthen core ROI technology by addressing stability and performance issues to enhance speed, usability, and efficiency.
	10	**IT Support:** Strengthen technology support systems to improve responsiveness, uptime, and processor productivity.
	11	**Digital Enablement:** Support digital efforts relating to 3, 6, 7, 8, 13 and 14
GO-TO-MARKET	12	**Redesign Customer Incentives:** Explore alternative incentives that better incentivize digital or global adoption
	13	**Account Management:** Build a unified, technology-enabled account management model I to create consistent and proactive customer management
	14	**Contract Management:** Centralize and standardize contract management with clear governance, digital tracking, and defined triggers to align renewals and incentives with digital and global adoption goals.
PERFORMANCE MANAGEMENT	15	**Performance Management:** Redefine performance management and productivity metrics to provide a holistic view of efficiency and quality. Implement consistent performance expectations and consistent reporting and tracking.

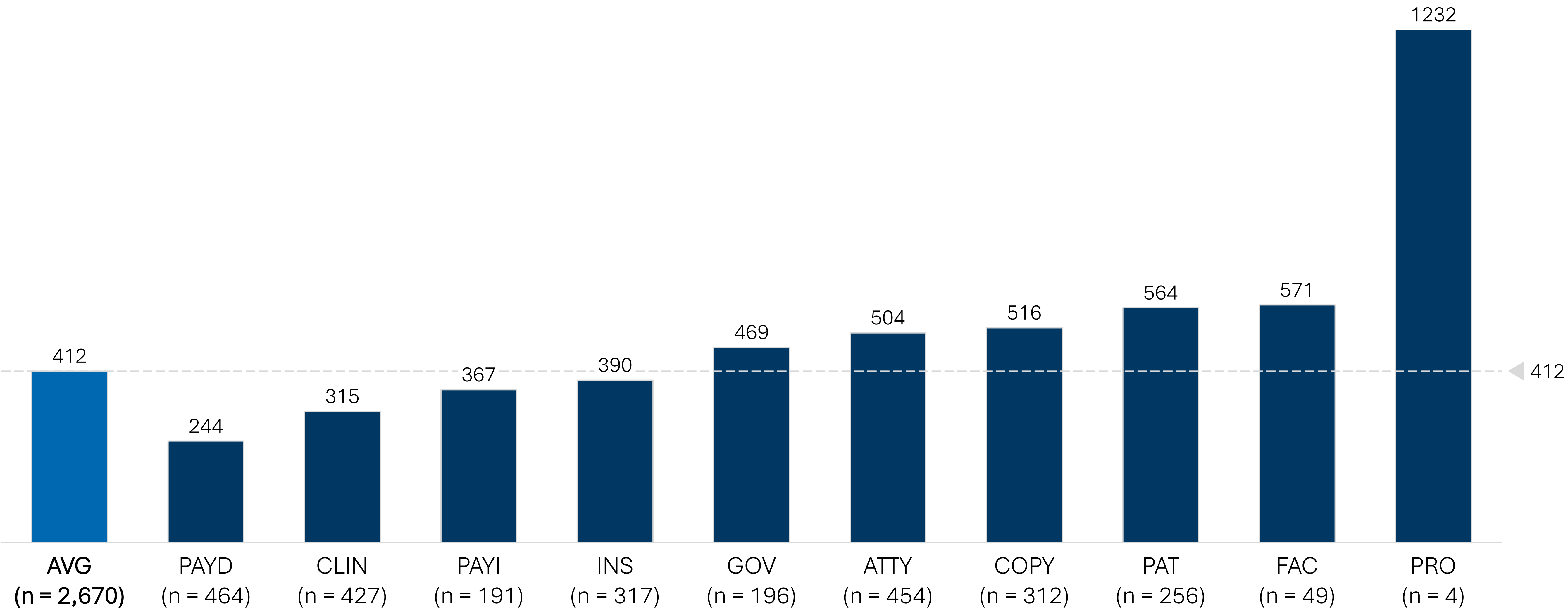
Next steps

- Develop solution deep dives, including organization model options
- Consolidate technology opportunities
- Develop prioritized roadmap and implementation structure

Appendix

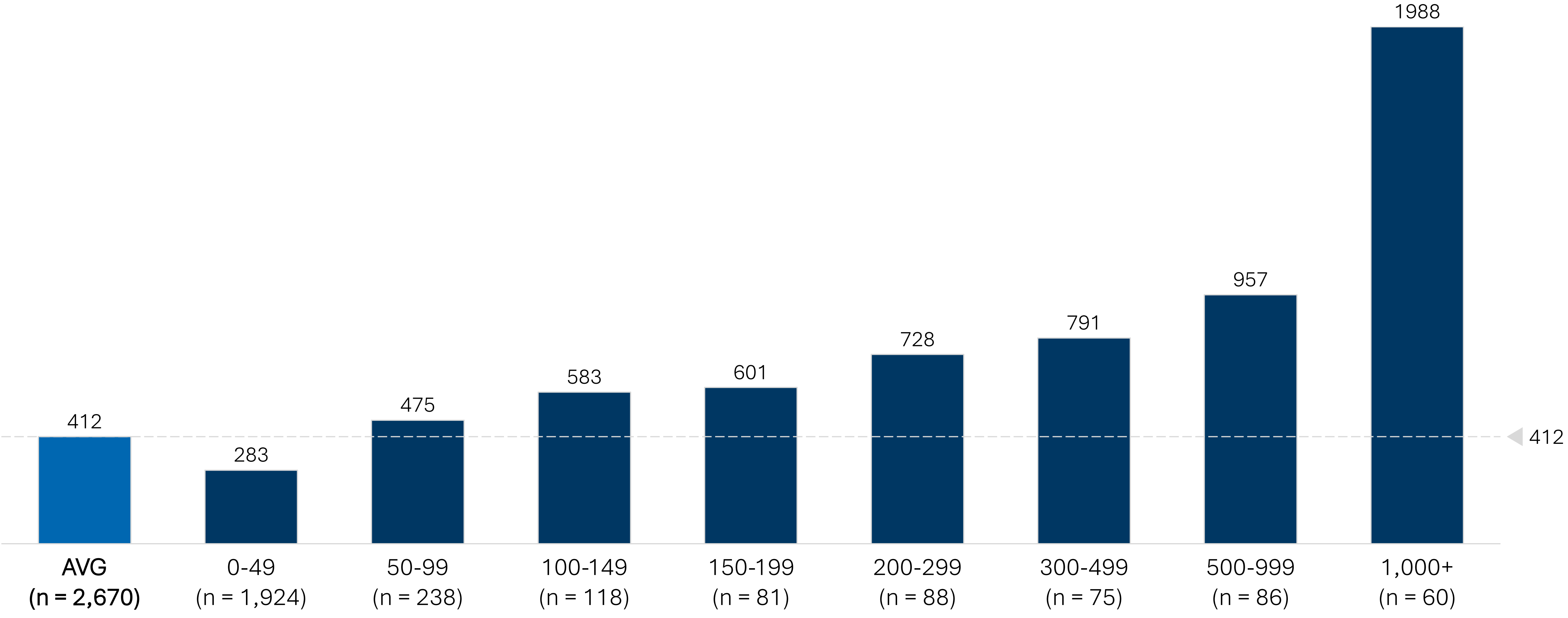
Detailed Time Study Data

Average Lead Time (sec) by Major Class



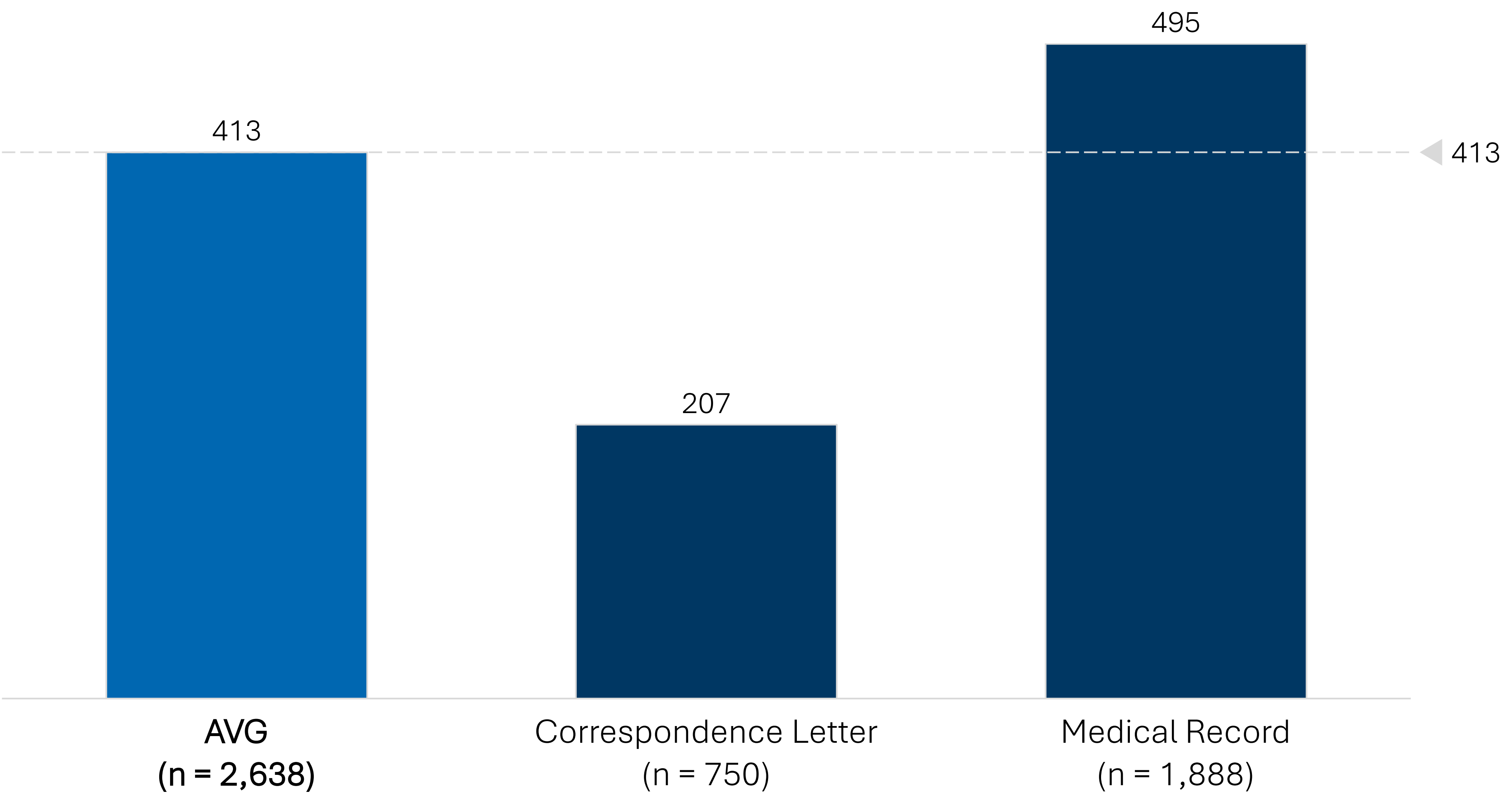
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count



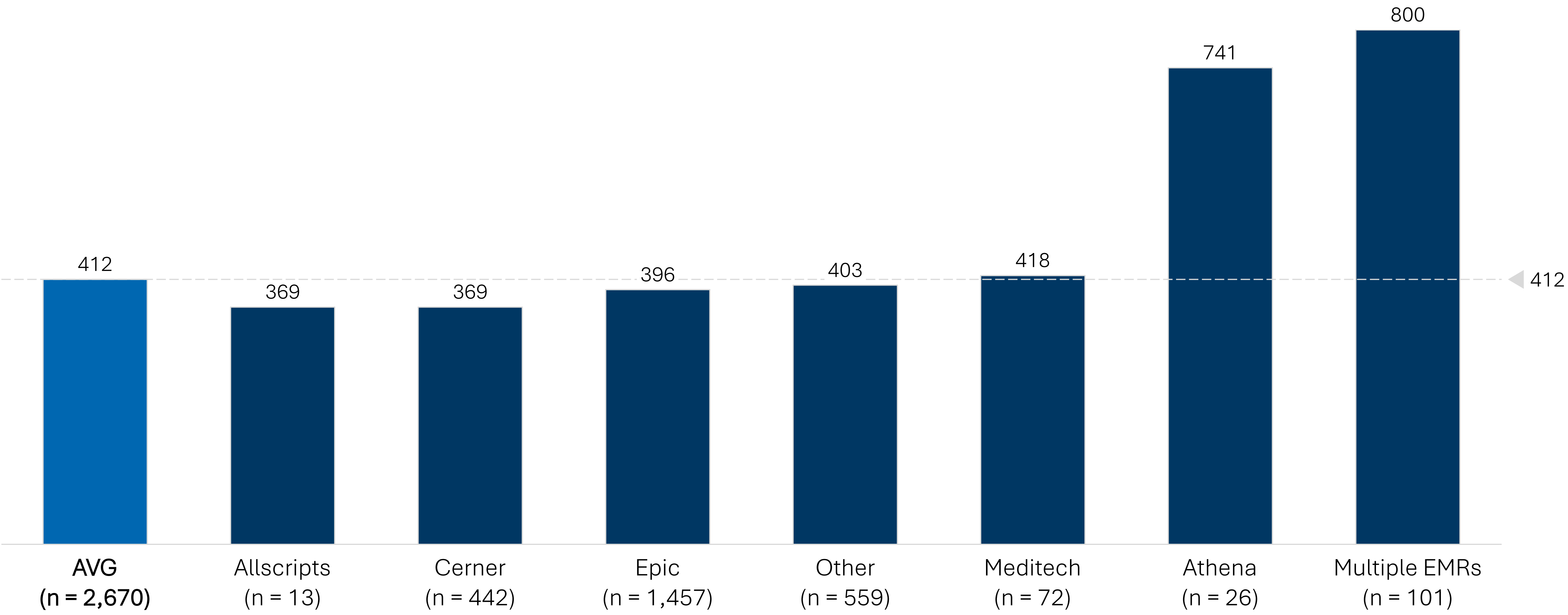
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Record Type



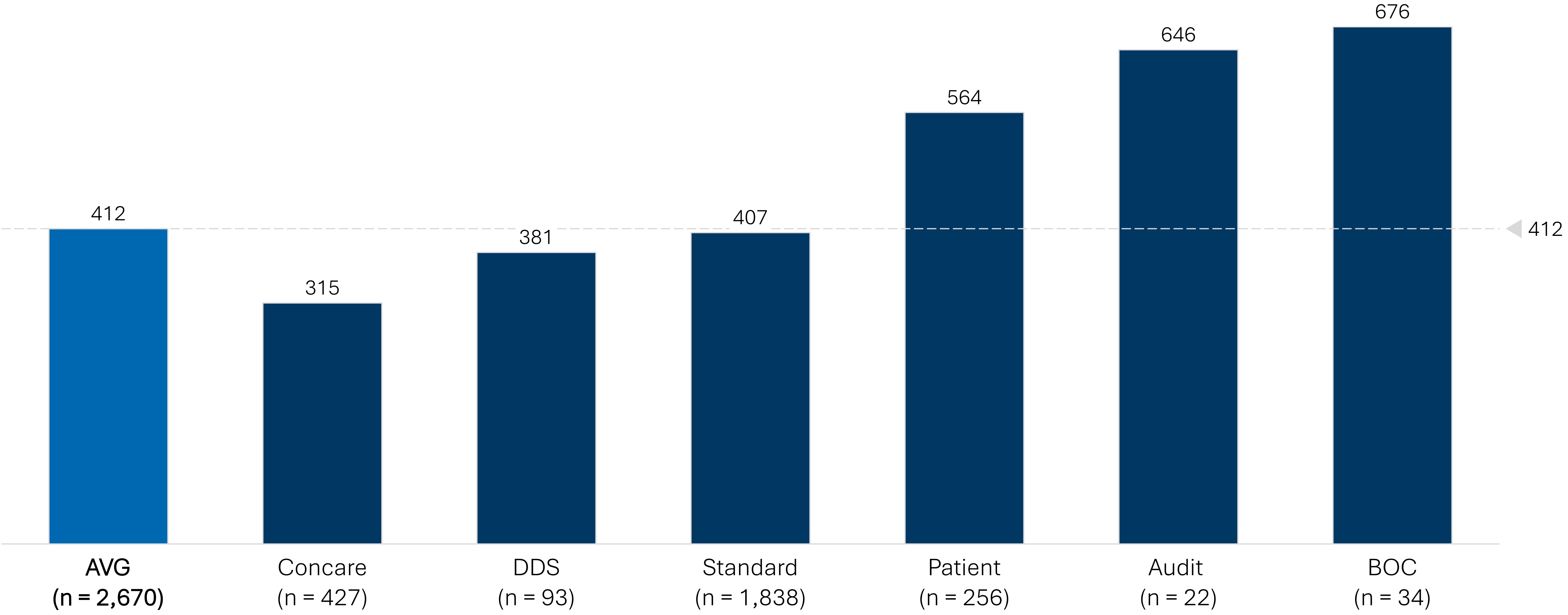
Note: Representative of non-DAR sample size excluding blanks (n = 2,638), as DAR requests can skew lead times

Average Lead Time (sec) by EMR



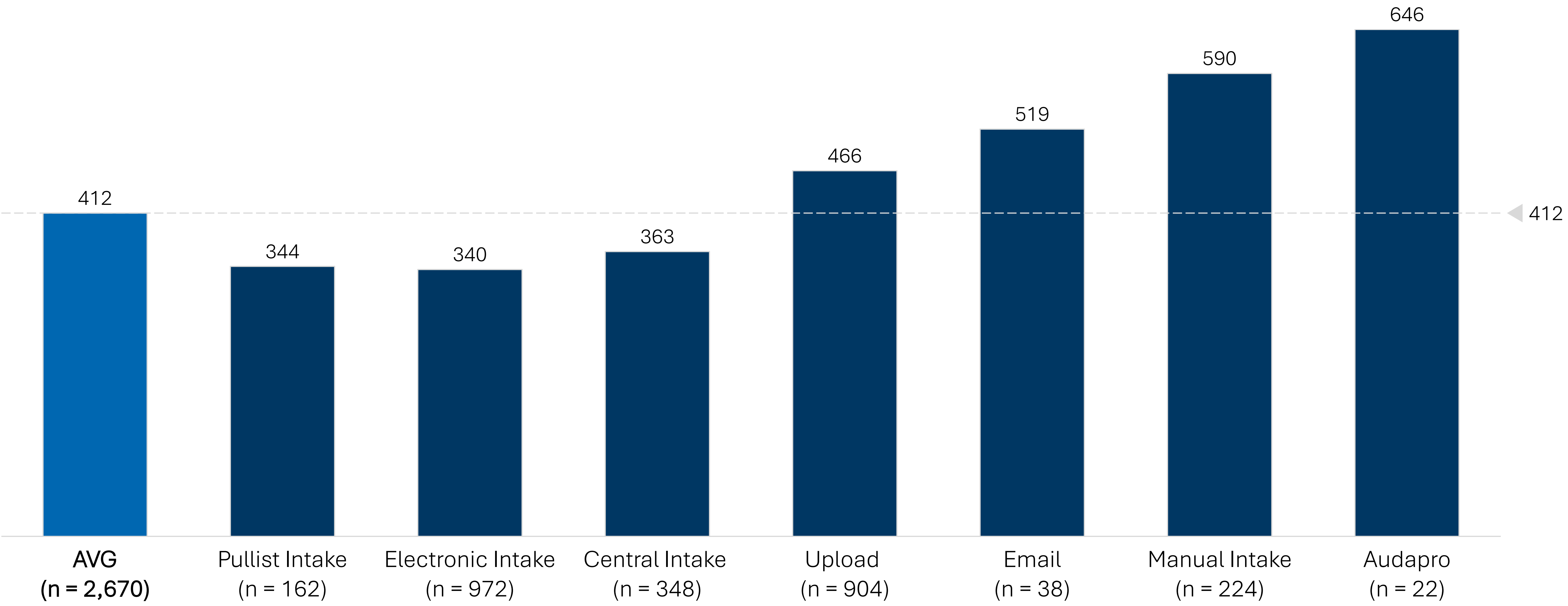
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Request Type



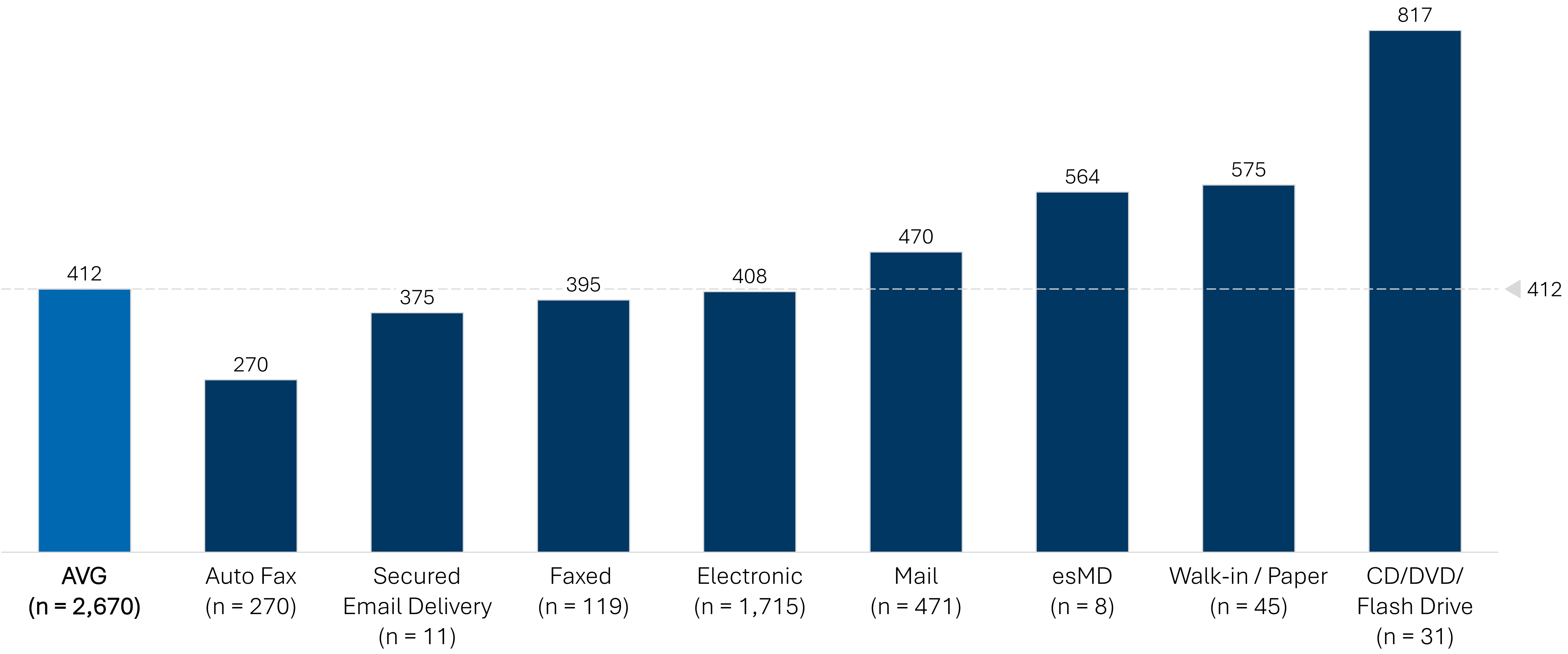
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Intake Type



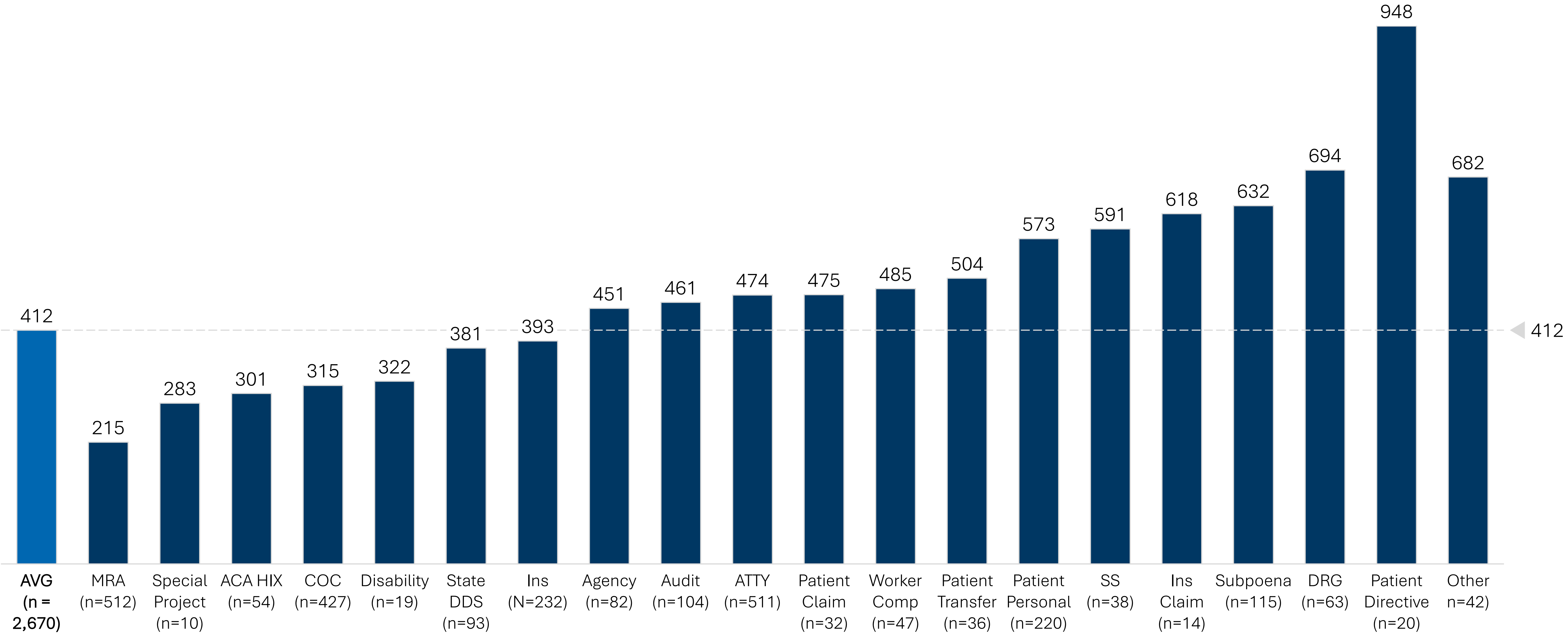
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Delivery Method



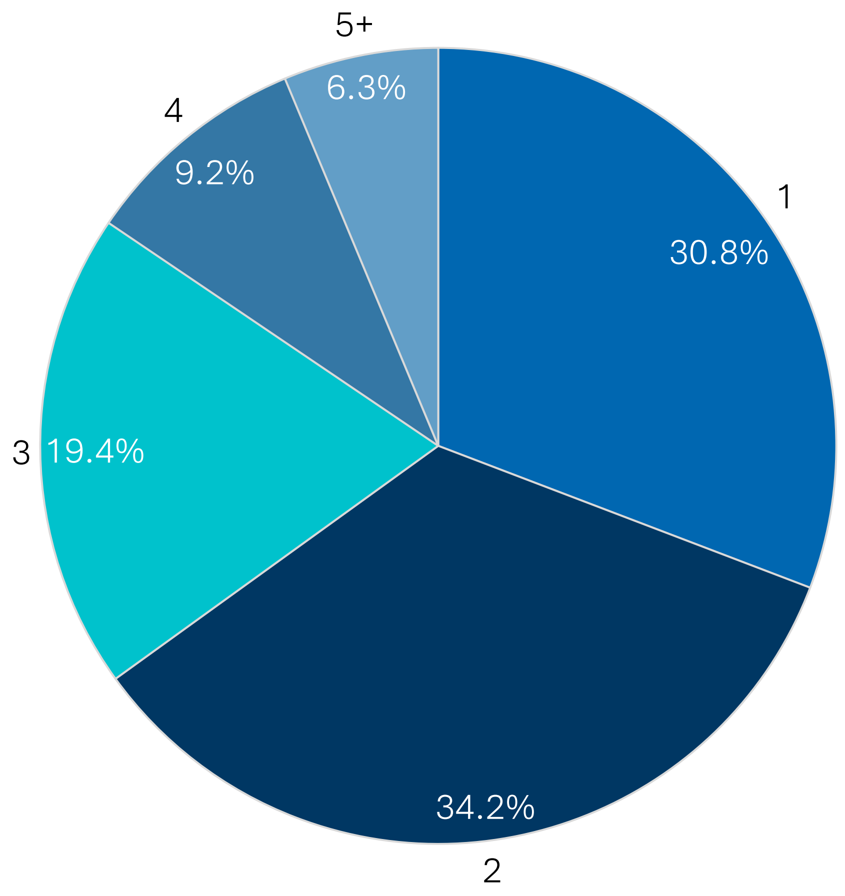
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

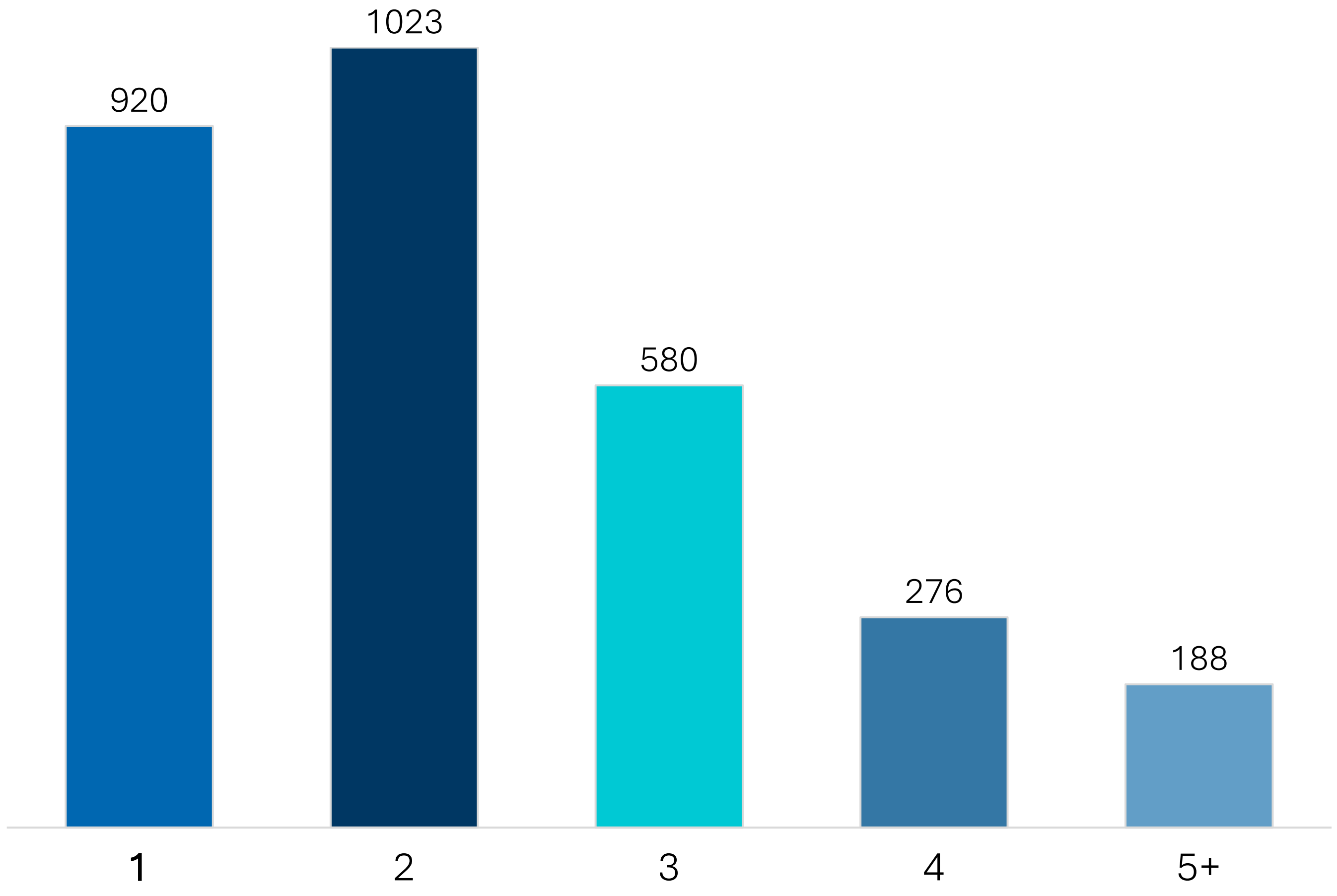
Average Lead Time (sec) by Primary Reason



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

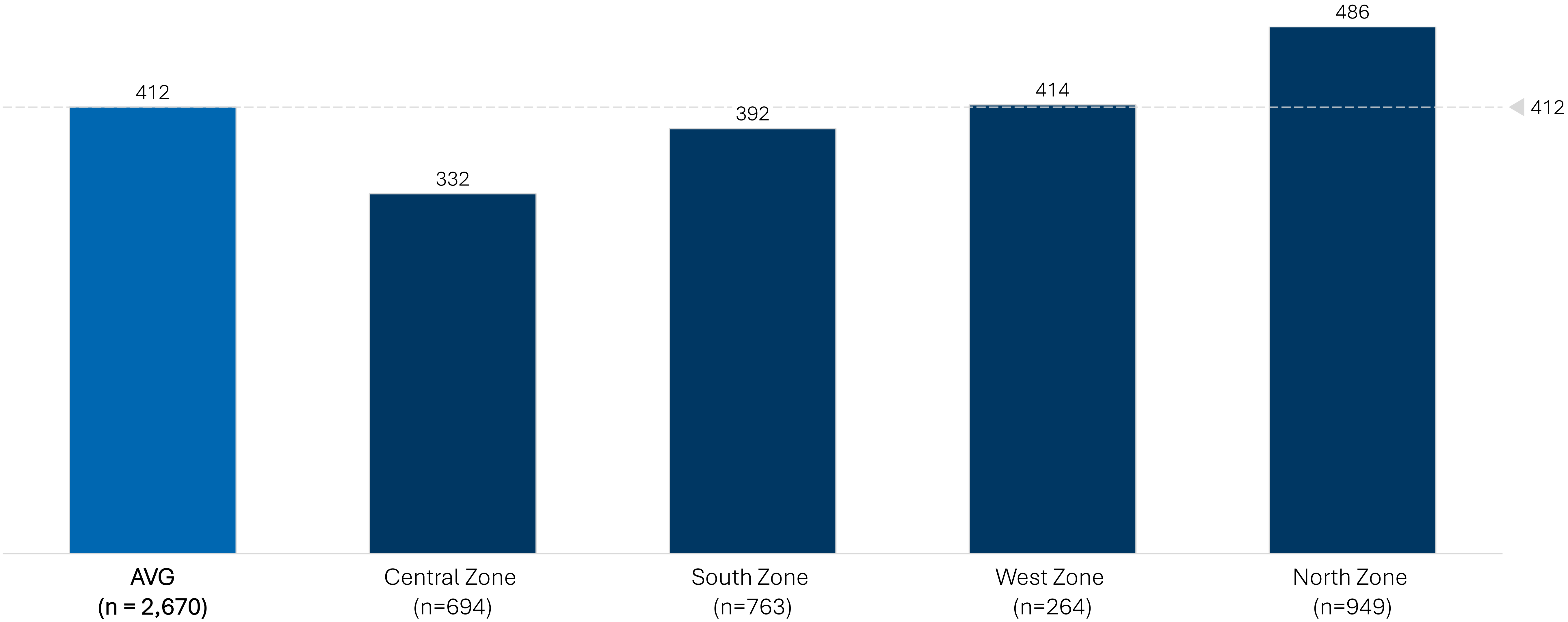
Sample Size by Number of Processors in Audit Log





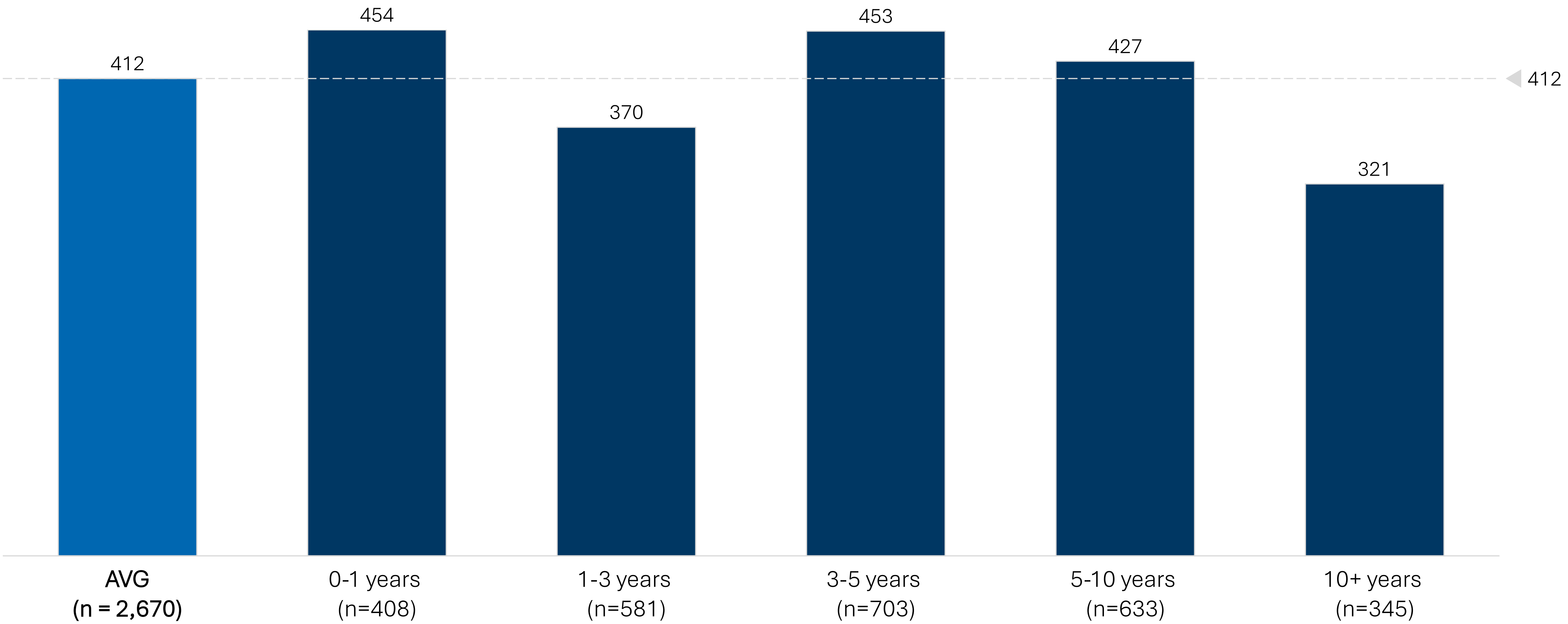
Note: Representative of complete sample size excluding 0 values (n = 2,987)

Average Lead Time (sec) by Zone



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Tenure of Processor



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

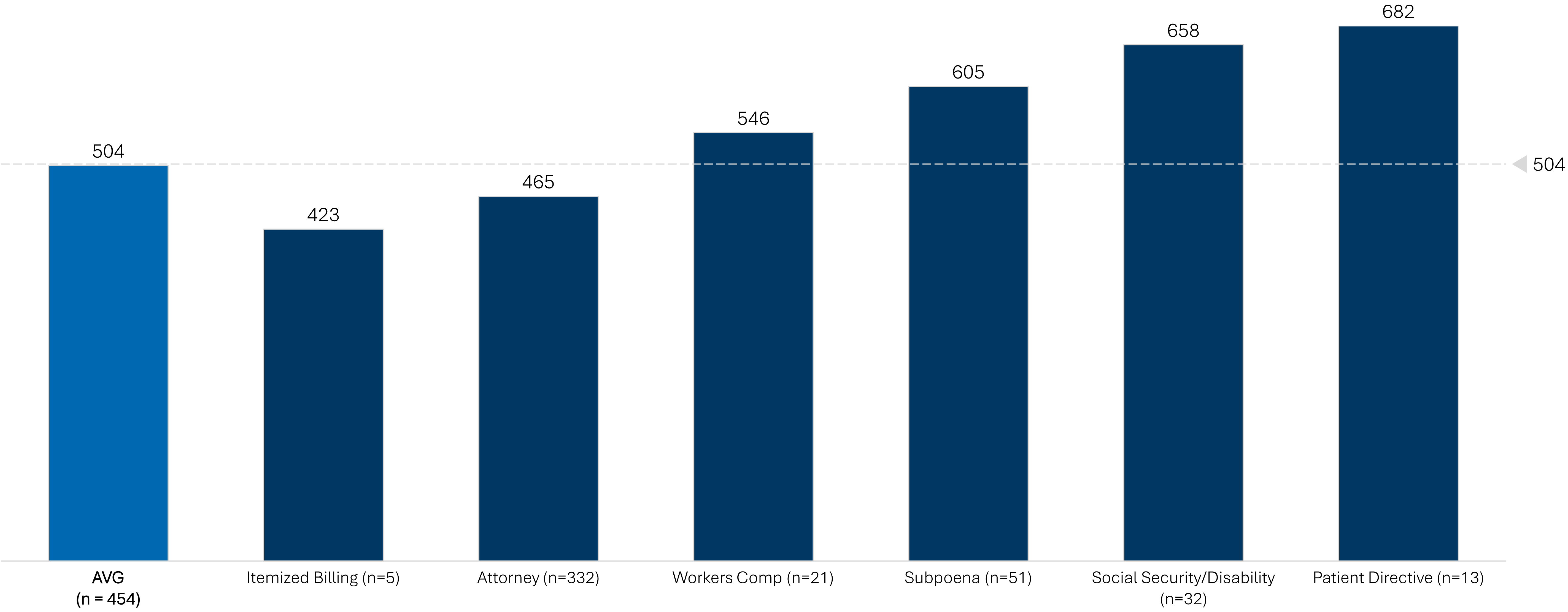
Processor Tenure Compared to Employee Census

	Employee Census Distribution	Total Processors Studied	% of Total Processors Studied	Variance to Census
New Joiners	17.8%	28	15%	-3.0%
1-3 Years	23.5%	36	18.9%	-4.5%
3-5 Years	24.3%	59	31.1%	6.8%
5-10 Years	19.9%	47	24.7%	4.9%
10+ Years	14.0%	20	10.5%	-3.5%

Note: Processor name was mapped to employee data file to determine tenure at Datavant

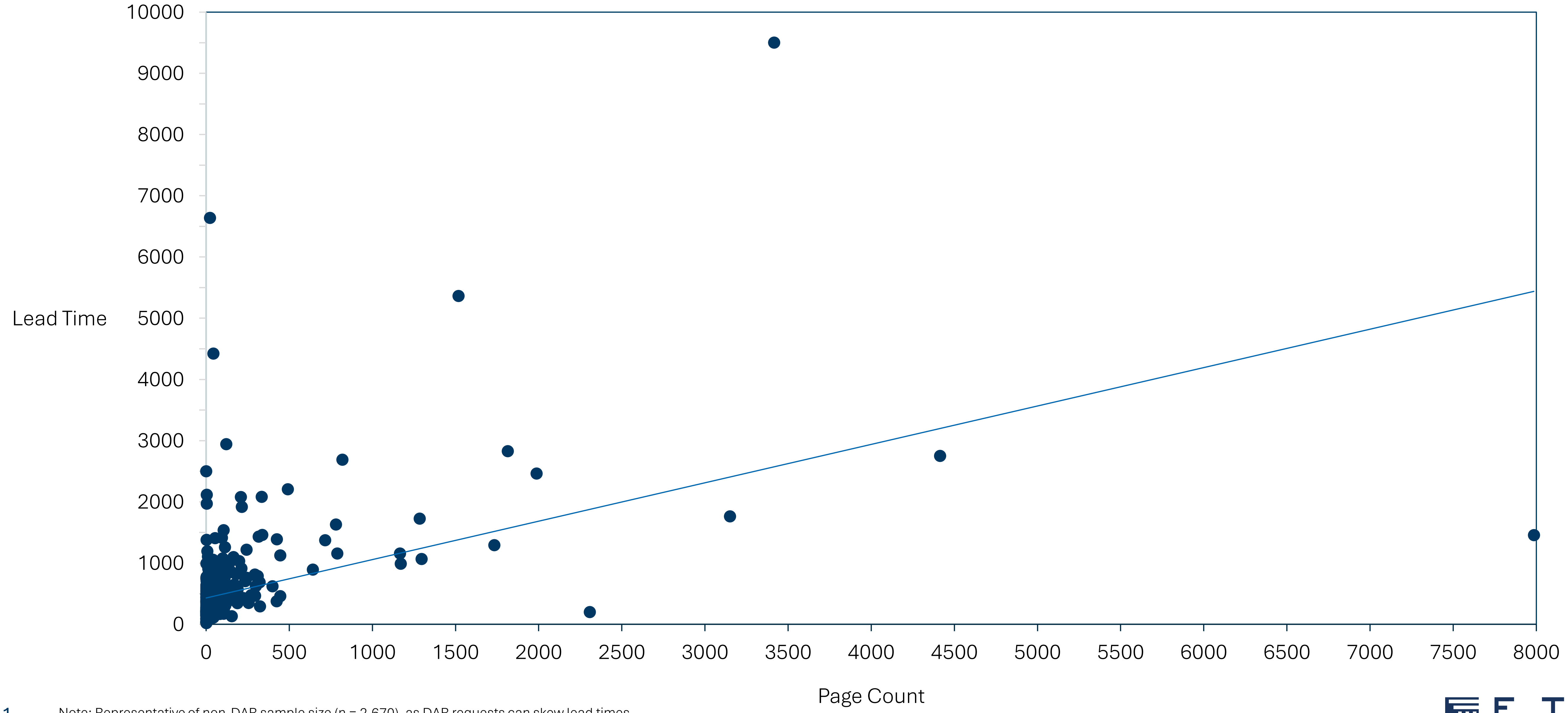
Supplemental Time Study Analysis

Average Lead Time (sec) by Primary Reason for ATTY Major Class



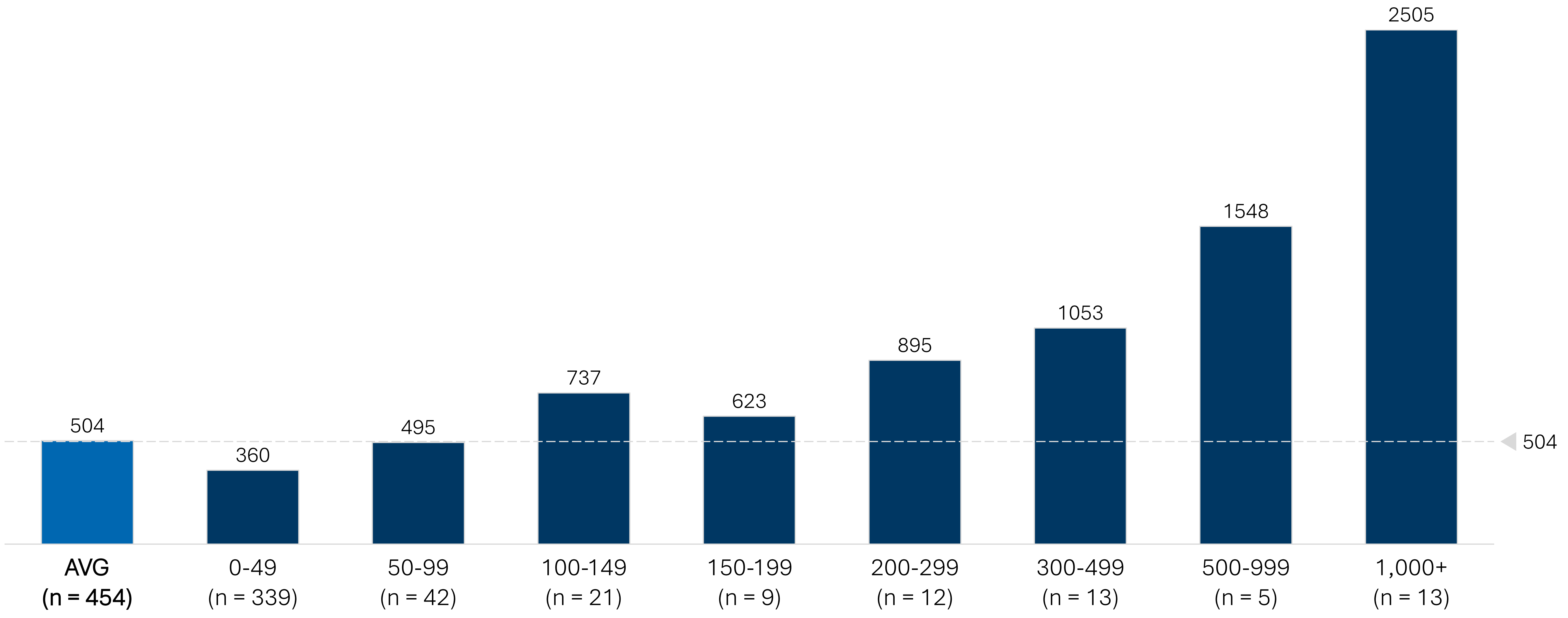
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for ATTY Major Class



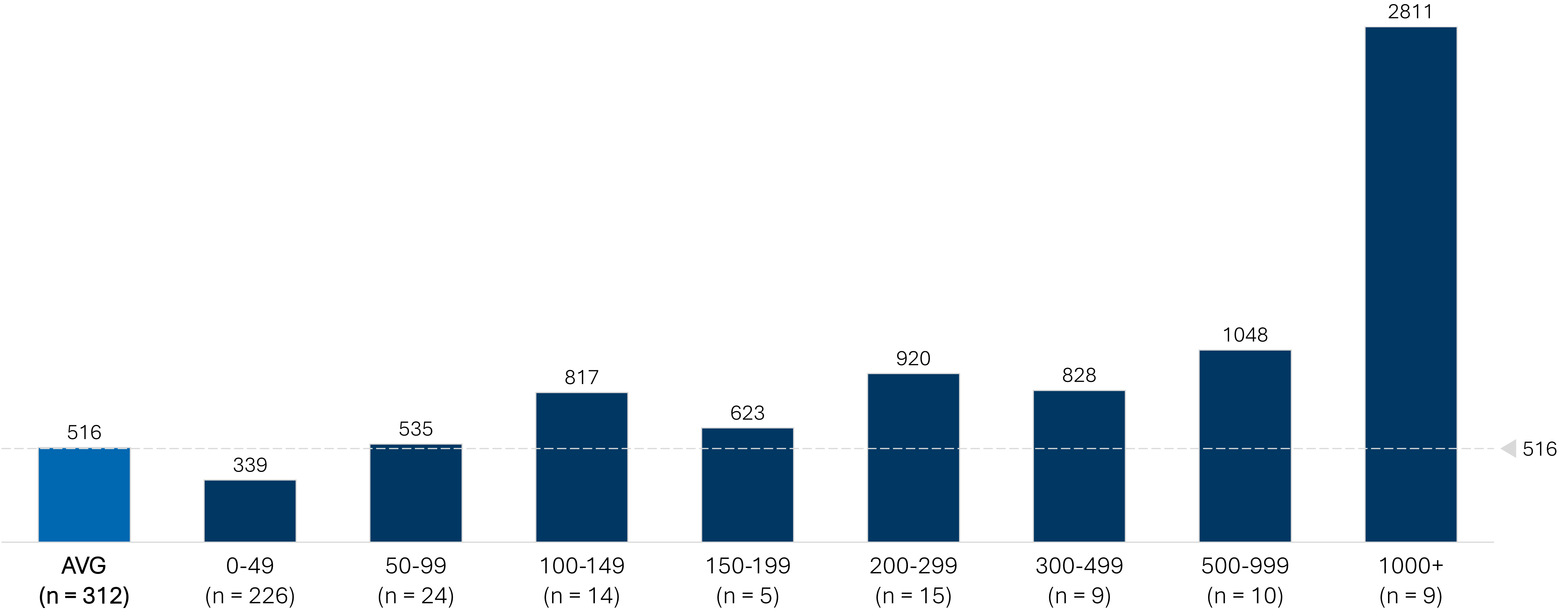
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for ATTY Major Class



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for COPY Major Class



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

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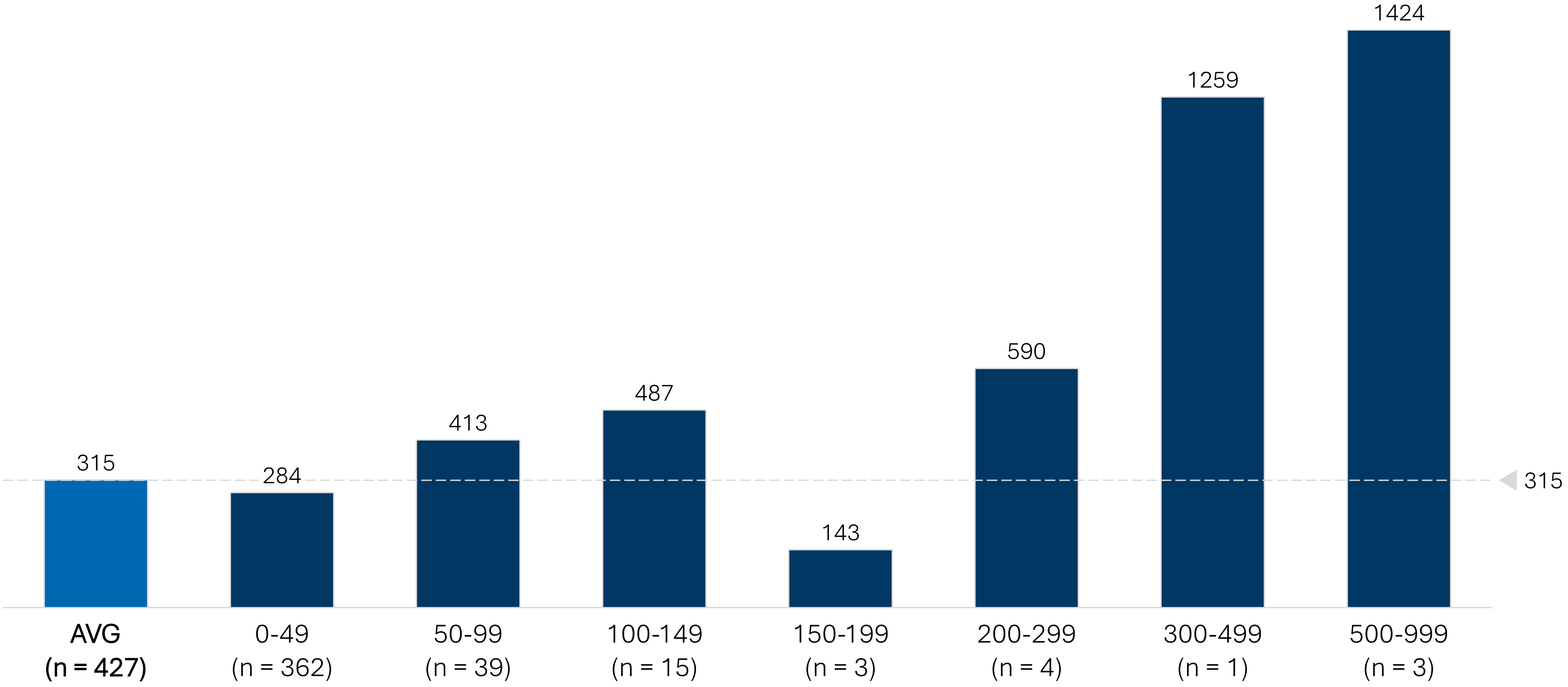
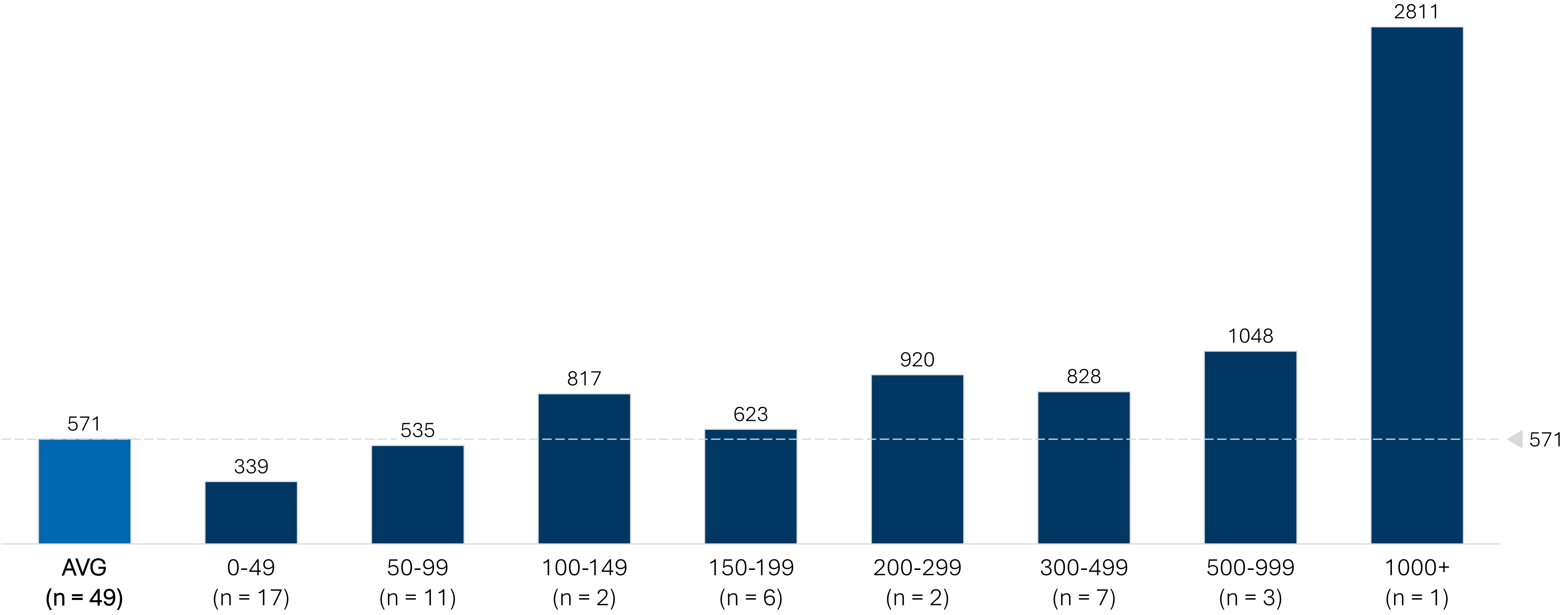
Average Lead Time (sec) by Page Count for CLIN Major Class



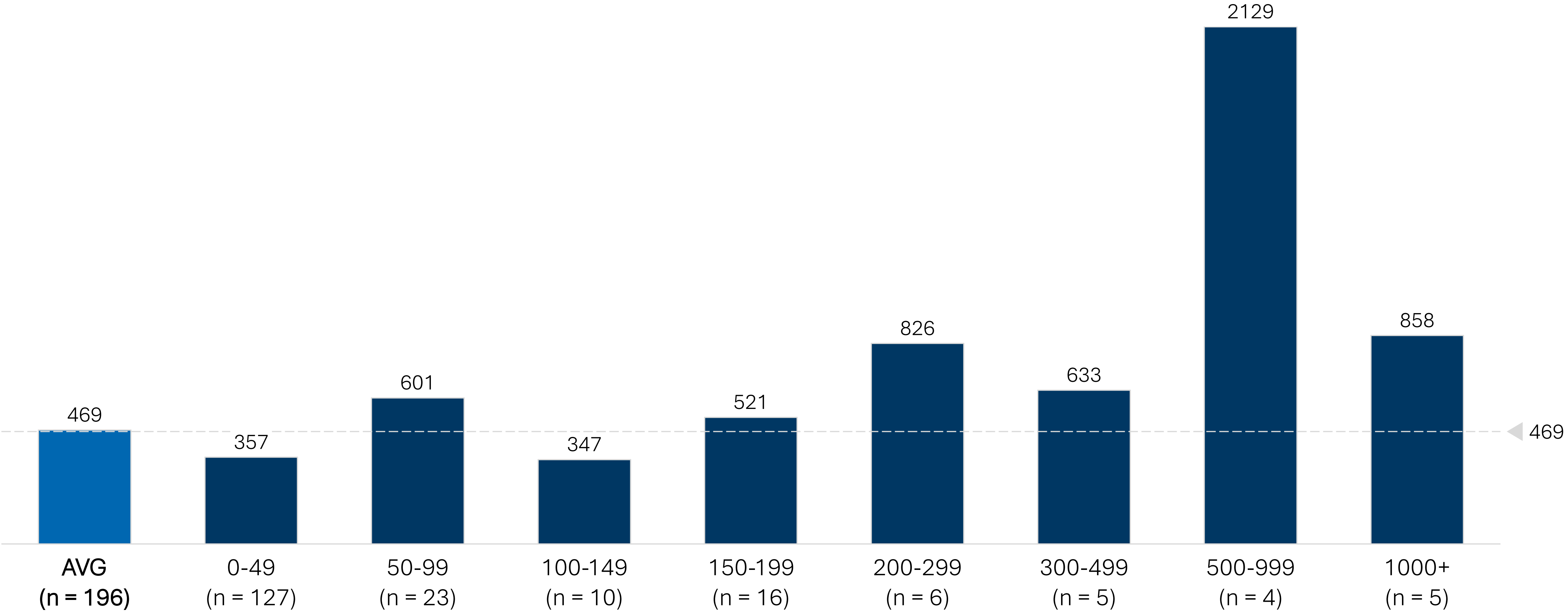
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for FAC Major Class



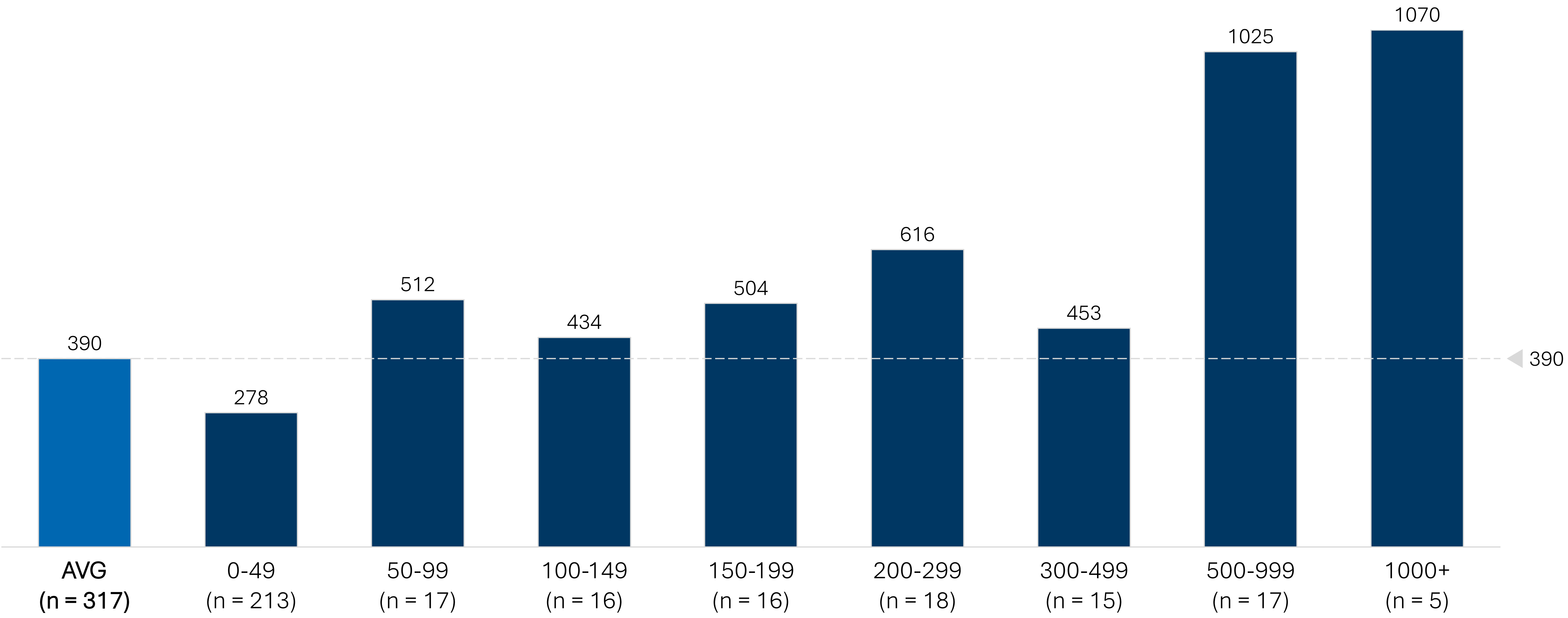
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for GOV Major Class



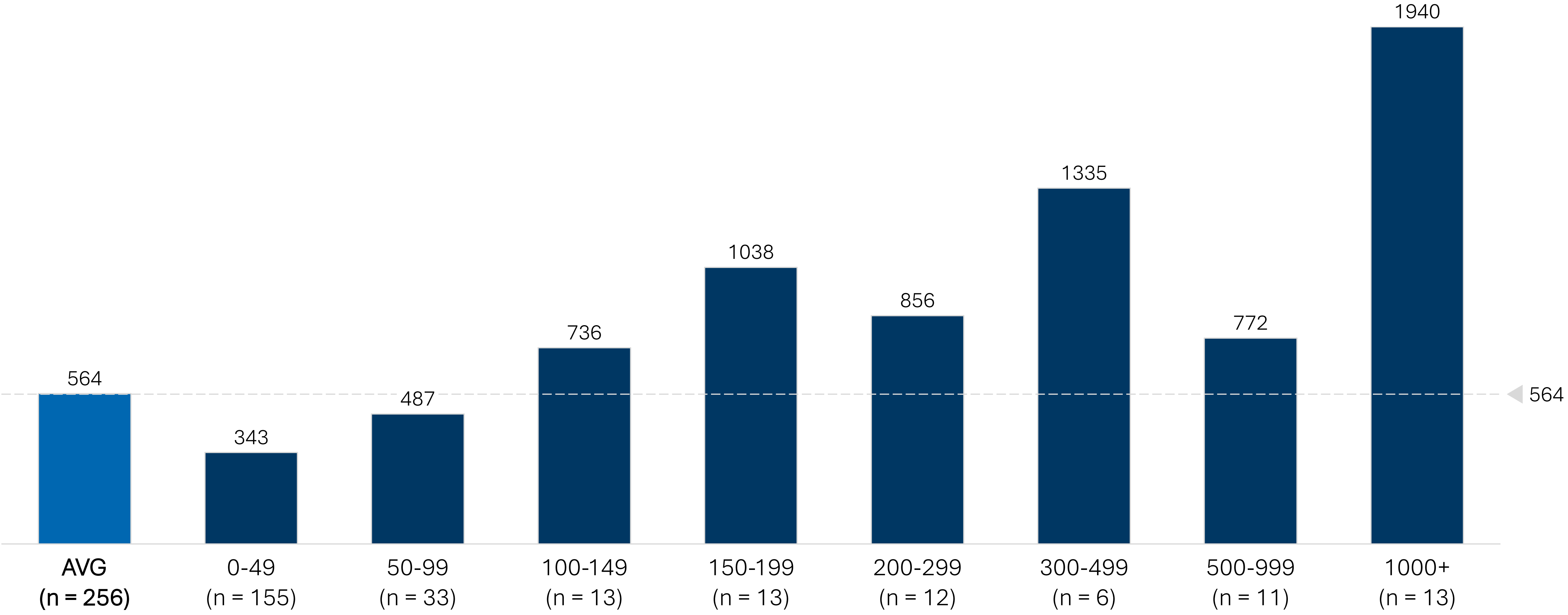
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for INS Major Class



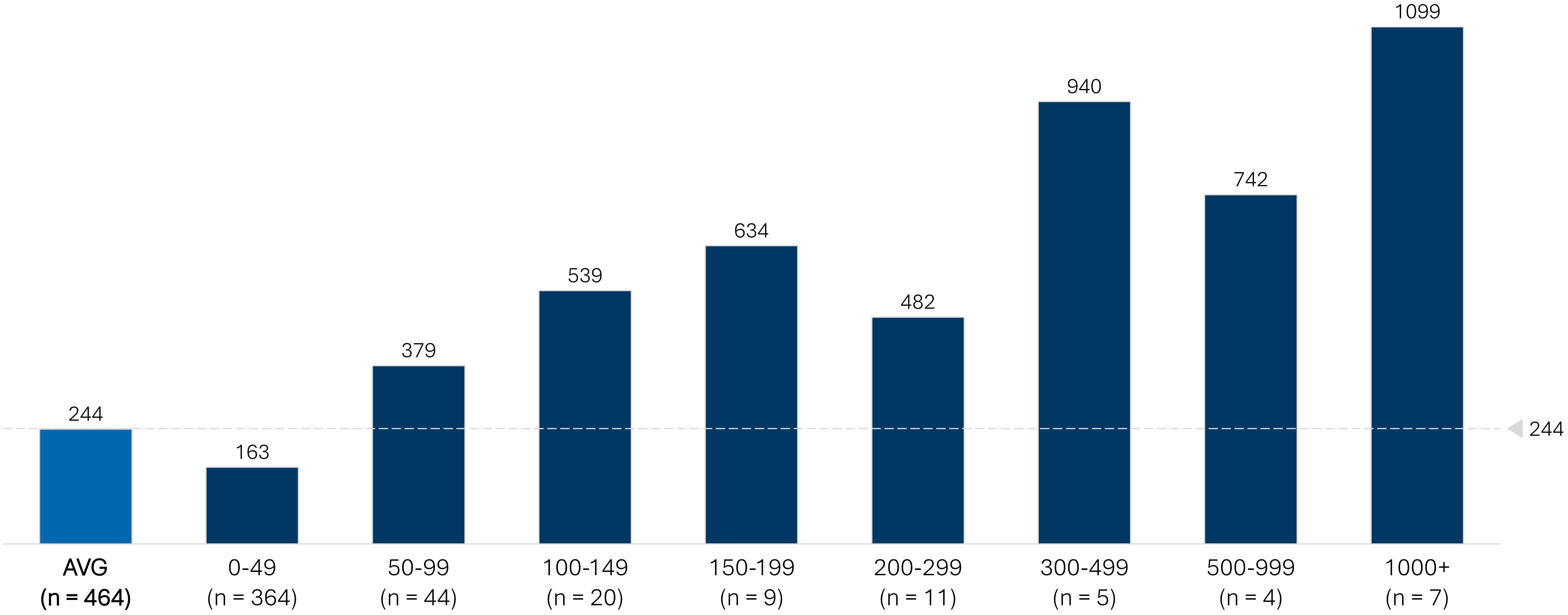
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for PAT Major Class



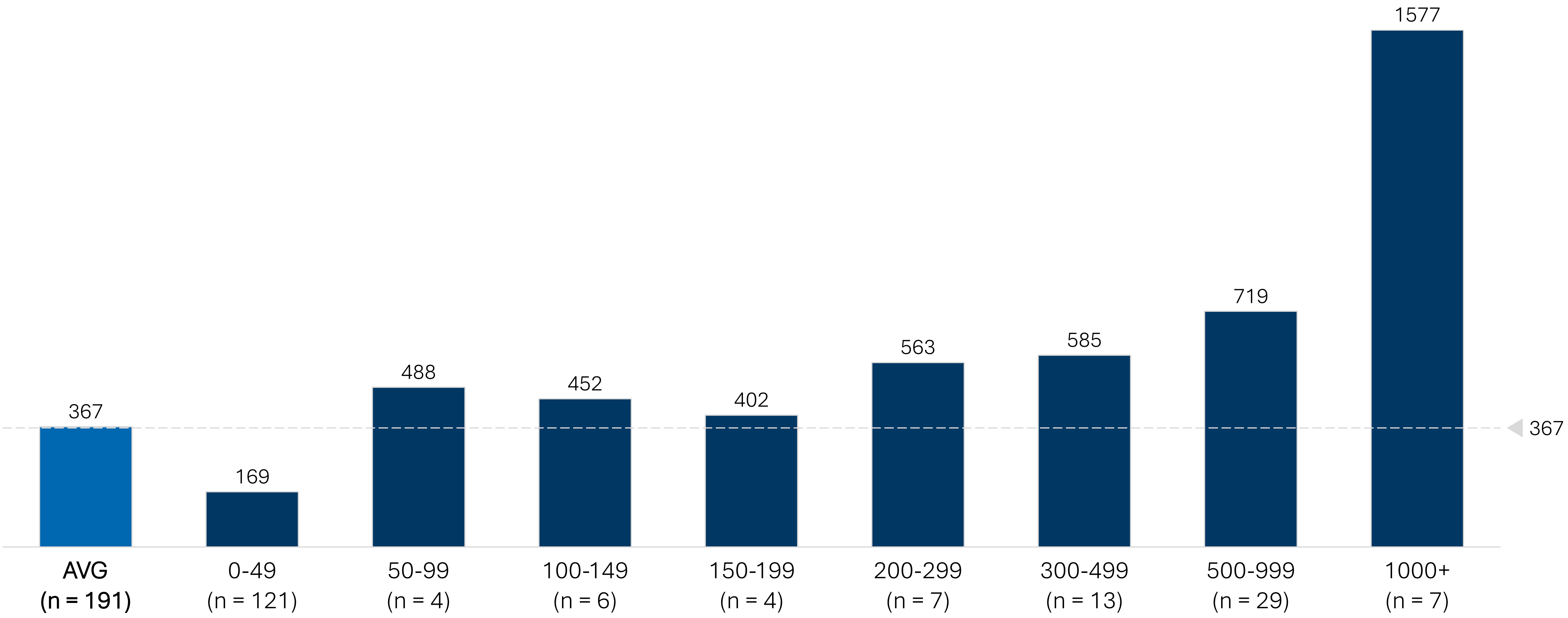
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for PAYD Major Class



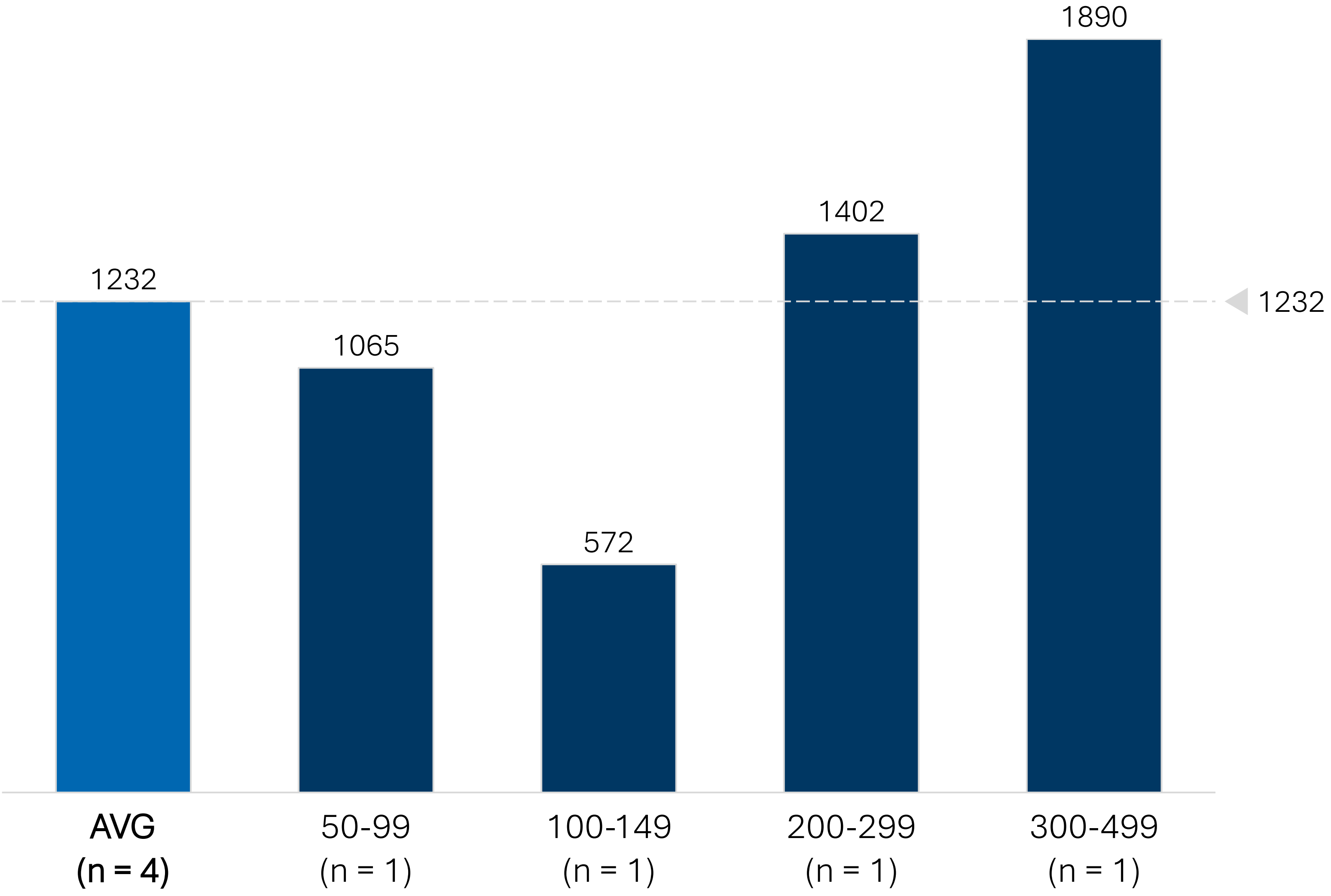
Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for PAYI Major Class



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Average Lead Time (sec) by Page Count for PRO Major Class



Note: Representative of non-DAR sample size (n = 2,670), as DAR requests can skew lead times

Additional Qualitative Follow-Ups

Challenges working with Athena (EMR)

- Our team observed 42 submitted requests:
  - 26 requests exclusively involved Athena
  - 16 requests involved Athena plus 1 EMR
- Our Athena sample represents performance from 8 processors with 1-10+ years of experience with Datavant
- One observer noted that a benefit of Athena is the processor can pull medical records and bills at the same time; however, other processors noted they were required to split records and billing requests into multiple EIDs for fulfillment
- Some observed requests were in ‘partially fulfilled’ status and the processors commented, “The system cannot automatically pull records from Athena, so I need to add them”
- One processor noted they had to select and print each visit individually from Athena (~50 visits in requested dates of service timeframe) which slowed productivity

High CL Rate for PAYI Major Class

- Our team observed 190 submitted PAYI requests, of which 90 requests resulted in a CL (~47%)
- Our PAYI sample represents performance from 25 processors across 19 health systems
- One processor received a bulk request for ~20 patients but was unable to find the specific provider at the health system; thus resulting in bulk CLs being sent out for each patient
- Another processor accounted for 68 of the total 90 CLs observed for the PAYI category; all of the requests were observed for a single health system (Cedars-Sinai Health Network)
- Due to the majority of CL volume being produced by two individuals, it is possible the high CL rate is more indicative of site-level challenges rather than problems within the major class

Observed Requests for PRO Major Class

- Our team observed 7 PRO requests:
  - 4 requests were submitted
  - 3 requests were cancelled (duplicates)
- Our PRO sample represents performance from 1 processor for 1 health system (HCA)
- The processor commented that her team focuses on ‘special projects’ and atypical requests (e.g., HPF as the EMR)
- One challenge she experienced working in HPF was when she accidentally clicked an incorrect record from the chart; she needed to restart the request from the beginning as it’s not possible to unselect within a request
- The page counts for the 4 submitted requests were 66, 103, 280, and 396 pages which likely contributed to the elevated lead times due to the small sample size

Detailed Pain Points & Surveyed Frequency

Detailed Pain Points – Overall

KEY
- OVERALL
- VIRTUAL
- ONSITE

RARELY — SOMETIMES — ALWAYS

Distributed team schedules across time zones creates collaboration gaps	Lack of access to Adobe causes export compilation difficulties	Serious tech issues	Unclear policies on when to QC, pend, or send correspondence letters	Variance in requests requiring a lot of homegrown knowledge	Systems outright not working	HS uploads are often slow, inconsistent and fail midway
Significant time spent managing and reviewing emails	Unclear guidelines for special cases / sites requiring specific knowledge	Long wait times for IT to support issues	Limited immediate access to supervisors necessitating escalations	Variability in home network quality affecting HS load times	Productivity metrics not considering request variance	Variable methods for productivity tracking and task assignment
	Inefficient office layouts require extra time to fulfill requests	Service creep onsite without managers at each site	VDI slows down the entire retrieval process	Sub-optimal remote work setups	Lack of general SOPs across sites	Demanding onsite multitasking requirements impacts focus and efficiency
			Remote environment is more susceptible to compliance risks	Remote troubleshooting takes a lot longer		Unpredictable workload: upcoming requests and in-person tasks
				Onsite dynamics / distractions effect traditional productivity		

Detailed Pain Points – Intake

KEY

- OVERALL
- VIRTUAL
- ONSITE

RARELY — SOMETIMES — ALWAYS

- For one record, must break into multiple EIDs as billing bot only takes 12 DOS at a time
- Inefficient HS comments / audit log forces full scrolling for each review
- Time spent rerouting requests that are sent to the wrong site
- Some requests require mailing charts from Iron Mountain to the facility before Fulfillment
- Outdated letterhead on requests leads to misrouted requests
- Unsuccessful handoffs between onsite employees and medical records department

- HS does not recognize billing requests or put it in a separate queue causing rerouting
- Verification challenges from obstructed scans
- Misaligned incentives between accurately fulfilling requests and hitting metrics
- Referencing the facility site list with each new request to verify permissions
- Unclear prioritization conventions lead to stat request overload
- Realignment often required to correct HS request queue

- Restrictions due to auth or type of record requested prevents release of records
- Other people frequently in requests processors open
- Requests are illegible
- Easy to game the system map factors and cherry pick requests to work on

- System fails to complete identified DAR requests
- Patient requests frequently missing critical fields or authorization to complete
- Searching for and identifying specific records for patients with many DOS
- Resetting HS parameters and filters in between requests

- Reliance on CLs to fulfill requests as follow-ups are challenging
- Onsite experience varies sit to site given there are no SOPs for sites

- Note pads, sticky notes, email drafts used as workarounds for HS
- Information on the requests is incorrect
- Lack of access to the necessary EMRs
- Work assignment approach varies and is often manual
- Highly variable request receival formats to monitor and log
- Lengthy bulk/FAX processing time
- Walk in processes are inconsistent and unpredictable which conflicts with targets

- Searching for and identifying specific records for patients with many DOS
- Duplicate recognition is inaccurate and time consuming in HS
- Request length dictates system load time
- Workflows differ across sites and teams
- Initial data entry from logging team / STORK is incorrect
- Mail workflow is extremely manual
- Walk in requests requires logging every factor on HS

Detailed Pain Points – Fulfillment

KEY

- OVERALL
- VIRTUAL
- ONSITE

RARELY — SOMETIMES — ALWAYS

Some requests are fulfilled entirely in the EMR without being logged in HS	No indicator if a certificate is required in HS which causes more manual work	Some requests require multiple EMR logins which reduces productivity	EMR access issues and login timeouts	Multiple systems and site-specific processes limits processors' ability to support across queues	Challenges sourcing old medical records
Lengthy record requests require additional effort to transmit to requestor	Bills must be saved individually in EMRs, reducing efficiency	Frequent distractions can lengthen request lead times	Many requests must be rerouted to CT or Radiology increasing lead time	The billing records download process is manual	Manual and inconsistent document export processes
Workarounds needed to deal with the finicky billing bot			Duplicate actions are performed in HS and the EMR	Missing EMR filters and bulk-select require fill chart review for each request	Realignment often required to correct request queue
Billing bot takes awhile to work through requests which slows productivity				Some EMRs can only download and fax a few pages at a time	Correspondence workflow is very manual and CI template has insufficient detail
DAR request requires multiple touchpoints				EMR processing and download times are prolonged when using personal Wi-Fi	Significant wait times when downloading larger files
				Pulling records into HS in 250-page increments for secure capture is time consuming	Patient records are not available or cannot be found after processor searches for it
				Can't directly print from EMR into HS so have to download to PDF and then upload	Patients request any and all records which takes a lot of time to go through

Detailed Pain Points – QC and Delivery

KEY

- OVERALL
- VIRTUAL
- ONSITE

RARELY — **SOMETIMES** — **ALWAYS**

Variability in contract management prevents operational efficiencies	The process to review sensitive information is manual and error prone	Additional QC checklists and templates across sites increases already lengthy QC process	Product enhancements do not always fit with the processor's workflow	Significant delays and crashes when uploading files into HS	HS lacks bulk upload functionality
Over-cautious approach to QC causes pended requests that could be filled	Travel time to FedEx to mail and ship completed requests disrupts workflow	Duplicative requests stemming from correspondence workflow		Excessively tedious QC process that hurts processor productivity	No direct HS / EMR integration makes the workflow more challenging
Difficulty in determining when to pend, escalate requests, or send CL		Cannot distinguish partially vs. fully fulfilled requests for multi-DOS cases in HS		HS Verify Assist is often incorrect or fails when dealing with large requests	Blank pages have to be manually deleted from documents
Unnecessary escalations and incorrect exception flagging		Productivity suffers due to access issues that ebb and flow with demand		Faulty onsite equipment leads to printer issues	
				Additional time is needed to generate PACS imaging requests on CDs	

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